CAC MASTERS SERIES

CAC. 701

FUNDAMENTALS OF COUNSELING THEORIES

MASTER IN CHILDREN AND ADOLESCENT COUNSELING

TRIUNE BIBLICAL UNIVERSITY GLOBAL EXTENSION USA

FUNDAMENTALS OF COUNSELING THEORIES

CAC. 701

Fundamentals of Counseling Theories

CAC. 701 · First Semester · 1.6 units · A complete lecture course for a sixteen-week semester

TRIUNE BIBLICAL UNIVERSITY GLOBAL EXTENSION USA

First Edition · 2026

Student Handbook of Comprehensive Lectures
Master in Children and Adolescent Counseling

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DEDICATION

To the children and young people who sit in the room and are not yet sure whether they can be helped —

and to the counsellors who learn to be worth trusting.

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About This Book

This volume is the complete lecture course for **CAC. 701 Fundamentals of Counseling Theories**, a First Semester core course of the Master in Children and Adolescent Counseling, carrying 1.6 units. It is written to be taught from and studied from without any other material in the room.

What the course is for

Every other course in the programme will teach you something to do with a child or an adolescent. This one teaches you why doing it might work, and how you would know whether it did. A counselling theory is not a belief system and not a loyalty; it is a set of answers to four hard questions — what a child is, what maintains the difficulty, by what mechanism change occurs, and what the counsellor therefore does. Answered well, those questions make practice deliberate, teachable and defensible to the parent, the school, the court or the licensing board that asks what you did and why. Answered badly or not at all, practice becomes a series of intuitive moves that cannot be evaluated and cannot be revised when they fail.

The course moves through the four great lineages of counselling theory — psychodynamic and attachment, behavioural and cognitive, humanistic and existential, systemic and family — reading each against the same four questions, and ends where practice actually lives: in the integration of competing accounts into a formulation for one particular child, held to the standard of the available evidence.

How the semester is organised

Sixteen weeks. Weeks 1 to 14 teach. Week 15 integrates: a comparative practicum in which the whole course is applied to a single extended case. Week 16 consolidates for examination. Each week is one lecture unit of about four notional hours — roughly ninety minutes of scheduled teaching, ninety minutes of seminar or practical work, and the remainder your own reading and writing. Each week has the same shape, and the shape is deliberate: outcomes you can be examined on;

the place of the week in the argument; four key concepts each tied to something a counsellor does in a room; three lectures built from propositions rather than narrative; at least one diagram placed where the argument needs one; worked examples; a practical with a debrief; seminar questions; the assessed task and its criteria; the reading, with the core text marked; and one sentence of synthesis.

A note on the child

The material is addressed throughout to work with children and adolescents, and that is not a decoration on an adult curriculum. A child cannot free-associate, may lack the vocabulary for a cognitive dispute, does not choose to attend, and is embedded in a family and a school that are themselves part of the problem and part of the solution. Theories built for adults fail in particular, predictable ways in a room with a nine-year-old. One purpose of this course is to teach you to see the failure coming.

Standards observed in this volume

British spelling in the book's own prose; American spelling preserved in quoted titles, journal names and direct quotations. Citations in APA 7th edition; only works actually cited are listed; no quotation, statistic or page number has been invented. All case material is fictitious and labelled as such; photographs generated for teaching are captioned as such; standardised instruments are described and attributed but never reproduced. Diagrams are drawn for this edition. The file is a tagged PDF conforming to PDF/UA-1. The governing specification is *HANDBOOK-STANDARD.md*, held with the series.

How to Use This Book

Read the outcomes first and last. Each week opens with what you will be able to do on completing it, written as capabilities that can be observed and examined. Read them before the week and again after it. If you cannot do what they say, you have not finished the week, however much of it you have read.

Work the examples as you meet them. Worked examples are set apart and are not optional reading. Cover the solution, attempt it, then compare. The distance between your attempt and the printed one is precisely the material the examination tests.

Do the practicals. Every week carries a practical — a timed exercise, a formulation task, a role-play, an observation or an analysis of supplied material — with a debrief. Most of them require another person or a recording of yourself and a clock. The practical is the part of the week most often skipped and the part that most changes practice. A theory you have only read is a theory you cannot use.

Keep the comparison grid. Figure 1.3 in week 1 is a blank grid: five questions down the side, four approaches across the top. Complete one column each time the course meets an approach, in your own words, and by week 15 reproduce the whole grid from memory and defend every cell. The grid is the course. Everything else is commentary on it.

Argue with the lectures. Each lecture ends in a clinical implication and a seminar question. The seminar questions are not rhetorical; write answers to them. Where a lecture states a contested position, the contest is named, and your task is to hold a position and say what would change it.

For the tutor. Each week is one four-hour block: ninety minutes of teaching, ninety minutes of seminar or practical work, the remainder preparation and follow-up. The three lectures are self-contained and may be reordered where a cohort needs the case before the theory. The assessed task printed in each week is the task set, and the criteria printed with it are the criteria used. Substitute local material in the vignette and the practical where a cohort's placements differ, provided the substitute is fictitious and labelled.

For the supervisor. The formulation exercises are yours. The question that converts knowledge into judgement is always the same: *which of the approaches in this week would formulate this case differently, and what would that formulation lead you to do on Tuesday?* A trainee who can state a mechanism of change and then say what would falsify it is ready for supervised caseload. A trainee who can only describe a theory is not.

Conventions

Convention	Meaning
Week *n*	the *n*th lecture week of the sixteen-week course
Lecture I, II, III	the three teaching sequences within a week
Worked example	a fully solved illustration; attempt it before reading the solution
Practical	a timed exercise with a debrief; requires a partner, a recording or supplied material
Hypothetical / fictitious composite	case material invented for teaching; no real client is described
Illustrative image (AI-generated)	a photograph created for this edition; depicts no identifiable person
Jurisdiction-dependent	a legal or reporting duty that must be checked against local law

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The Semester at a Glance

Sixteen weeks, one lecture unit each. Weeks 1–14 teach the course; week 15 integrates it on a single extended case; week 16 prepares the examination. The right-hand column names the assessed work of each week; the criteria for each are printed inside the week.

Week	Title	Principal lineage or task	Assessed
1	What a Theory Is For	the four questions; mechanism and technique; common factors	Comparative formulation, 1,500 words
2	The Psychodynamic Lineage I	Freud, Anna Freud, play as communication	Reading response, 600 words
3	The Psychodynamic Lineage II	attachment, the Strange Situation, the secure base	Attachment analysis of a vignette
4	The Psychodynamic Lineage III	mentalisation and reflective function	Reflective-function exercise
5	The Behavioural Lineage I	learning theory, contingency, functional analysis	Functional analysis of a case
6	The Behavioural Lineage II	cognitive therapy with children and adolescents	Cognitive formulation
7	The Behavioural Lineage III	third-wave: ACT, DBT-A, mindfulness-based work	Practical log
8	The Humanistic Lineage I	person-centred: the conditions and the child	Recorded micro-practice
9	The Humanistic Lineage II	existential and meaning work with adolescents	Essay, 1,200 words
10	The Systemic Lineage I	Bateson to Minuchin: structure,	Family map of a vignette

Week	Title	Principal lineage or task	Assessed
		boundaries, function	
11	The Systemic Lineage II	solution-focused and narrative approaches	Re-authoring exercise
12	Critique	multicultural, feminist and decolonial readings of theory	Critical review, 1,200 words
13	Integration I	case formulation across competing accounts	Formulation portfolio
14	Integration II	evidence-based practice and outcome monitoring	Outcome-measure critique
15	The Comparative Practicum	one extended case, four formulations, one recommendation	Practicum presentation
16	Revision and Examination	consolidation, past questions, examination technique	Final examination

Notional hours. 1.6 units ≈ 24 notional learning hours across the sixteen weeks, of which roughly one third is scheduled teaching and two thirds your own preparation, practice and writing. Weeks vary: the practicum week is heavier in supervised work, the revision week lighter in teaching.

Reading load. Each week marks one core text among its six readings. Read the core text before the week's teaching and the remainder across the week. The core texts of the course are Corey's *Theory and Practice of Counseling and Psychotherapy*, Geldard, Geldard and Foo's *Counselling Children*, and Weisz and Kazdin's *Evidence-Based Psychotherapies for Children and Adolescents*.

Fundamentals of Counseling Theories

SEMESTER	UNITS	CATEGORY
First	1.6	Core

Every other course in this programme will teach you something to do. This one teaches you why doing it might work, and how to tell whether it did.

A counselling theory is not a belief system and not a loyalty. It is a set of answers to a small number of hard questions: what a child is, what goes wrong, how change happens, and what the counsellor therefore does. Answered well, those questions make practice deliberate. Answered badly or not at all, practice becomes a series of intuitive moves that cannot be evaluated, cannot be taught, and cannot be defended to the parent, the school, the court or the licensing board that asks what you did and why.

The course proceeds in sixteen weeks. This week builds the frame. Weeks 2 to 4 take the psychodynamic and attachment lineage; weeks 5 to 7 the behavioural, cognitive and third-wave lineage; weeks 8 and 9 the humanistic and existential; weeks 10 and 11 the systemic, solution-focused and narrative; week 12 the critiques that keep all of them honest. Weeks 13 and 14 integrate: formulation and evidence. Week 15 runs the whole course over one extended case, and week 16 prepares you for the examination.

Throughout, the material is addressed to work with children and adolescents. That is not a decoration on an adult curriculum. A child cannot free-associate, may lack the vocabulary for a cognitive dispute, does not choose to attend, and is embedded in a family and a school that are themselves part of the problem and part of the solution. Theories built for adults fail in particular, predictable ways in a room with a nine-year-old, and one purpose of this course is to teach you to see the failure coming.

1 LECTURE UNIT 1 Teaching week 1 4 notional hours

What a Theory Is For

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **State the four questions** a counselling theory must answer, and identify which of the four a given written account of a case leaves unanswered.
2. **Construct a formulation** of a fictitious case that names the presenting problem, the factors maintaining it, and the mechanism by which an intervention would change those factors.
3. **Distinguish a mechanism of change from a technique**, giving one example of each from any two of the four major approaches.
4. **Evaluate a published effectiveness claim** using three tests: what the comparison condition was, who the population was, and what was measured.
5. **Explain a theoretical position to a parent** in ordinary language, without jargon, in under two minutes.

Outcomes 1–3 are examined in this week's assessed task; outcomes 4 and 5 are examined in the practical and recur in weeks 13 and 14.

Where this week sits

This week supplies the vocabulary that the remaining fifteen argue with. Nothing here is a theory of therapy; it is a specification of what any theory of therapy has to deliver. If you learn the four questions and the distinction between mechanism and technique, you will read weeks 2 to 12 critically rather than memorising them. If you do not, they will read as twelve lists of names. Begin the comparison grid (Figure 1.3) this week and keep it to the end of the course.

Key concepts

THEORY OF CHANGE

The account a practitioner can give of *why* what they do should produce the result they want. In the room: the sentence you could say aloud after a session — "I did that because I expect it to make this difference" — and the observation that would show you were wrong. A practitioner who cannot say the sentence is not being flexible; they are being arbitrary.

FORMULATION

A written or spoken account of a particular child's difficulty that connects history, maintaining factors and context into a single argument, and implies a treatment. It is the alternative to a diagnostic label: a label tells you what category the child belongs to, a formulation tells you what to do on Tuesday. In the room: the paragraph you could hand to a colleague who has never met the child, which would let them continue the work.

MECHANISM OF CHANGE
The process inside the client through which improvement is supposed to occur — insight, extinction of a conditioned response, corrective relational experience, cognitive restructuring, altered family structure, new meaning. In the room: what you are actually counting on happening in the child's head or in the child's relationships. Techniques are what you do; the mechanism is what you are relying on the doing to cause.

FIDELITY
The degree to which what was delivered matches what the method specifies. In the room: whether you performed the intervention the manual describes, or a comfortable variation of it. Fidelity matters for two reasons that pull against each other — a method delivered loosely cannot be evaluated, and a method delivered rigidly to a child it was not designed for may be useless or harmful. Holding both thoughts at once is part of the job.

Lecture I — Theory is not optional when the client is a child

Proposition 1. A child cannot be relied upon to tell you what is wrong, so theory is the instrument that supplies hypotheses where speech is unavailable. An adult client can, at least sometimes, report a pattern: *I always leave before I am left.* A seven-year-old with the same pattern cannot. Developmental limitations of vocabulary, memory, abstraction and metacognition mean that the child's account is partial, concrete, and often an account of what adults have said about them. Theory fills the gap by generating testable hypotheses from what is observable — play, drawing, refusal, somatic complaint, behaviour in the session — and by telling you what those observations would mean if the theory were right. This is the deepest reason child counselling is theoretical in a way that adult counselling can afford not to be.

Proposition 2. Every intervention is already theoretical; the choice is between an examined theory and an unexamined one. A counsellor who says they are "not theoretical, I just build rapport" has adopted a position with a mechanism (the relationship produces change), a theory of the problem (problems persist where the child does not feel safe), and a method (be warm, be consistent, do not push). It is a person-centred position, descended from Rogers, and it is defensible. What is not defensible is holding it without knowing it, because then it cannot be revised when it fails.

Proposition 3. Theory is what makes practice evaluable. If you have stated a mechanism, then non-improvement is informative: either the mechanism did not operate, or it operated and did not produce the predicted change, or your formulation of the problem was wrong. Those are three different findings and they

lead to three different next steps. Without a stated mechanism, non-improvement is only disappointment. This is the difference between a practitioner who learns from twenty years of caseload and one who repeats one year twenty times.

Proposition 4. In work with minors, theory decides who is in the room. A behavioural theory of a child's refusal to attend school locates the problem in contingencies and will want the parents and the teacher present. A psychodynamic theory locates it in anxiety and internal conflict and may want the child alone. A systemic theory locates it in the family's organisation and will want the whole family. These are not variations in style; they are variations in who you invite, whose account you privilege, whose consent you need, and whose interest you are serving when the child's interest and the parent's interest diverge — which, in work with children, they routinely do.

> **PRACTICE NOTE**
> **Clinical implication.** Before your next case, write one sentence of the form: *I am working from the idea that this child's difficulty is maintained by X, and that Y will change it, which I would expect to see as Z.* If you cannot complete the sentence, you do not yet have a plan; you have a schedule of appointments.

Worked example 1.1 — Finding the missing question

Task. A training brochure describes a method for childhood anxiety in these terms: "Our programme uses structured play sessions in which the child externalises the worry and the counsellor guides the child to re-author the worry story. Sessions are forty-five minutes, weekly, for ten weeks. Families report high satisfaction." Which of the four questions does this account answer, and which does it leave open?

Solution. It answers question 4 (method) completely — structure, duration, frequency — and gestures at question 3 (mechanism) with "externalises" and "re-author", which implies a narrative mechanism: changing the story changes the relationship to the problem. It does not answer question 2 (maintenance): nothing is said about what keeps this child's anxiety going now, so the method cannot be matched to the case. It does not answer question 1 (view of the child) at all, so there is no statement about which children the method suits or at what developmental level "re-authoring a story" is possible — a capacity most children acquire only in middle childhood. Two questions open, one half-answered: usable as a technique, not yet as a theory.

Lecture II — The four questions

Every counselling theory, whatever its vocabulary, answers four questions. Figure 1.1 sets them out around the case they exist to explain.

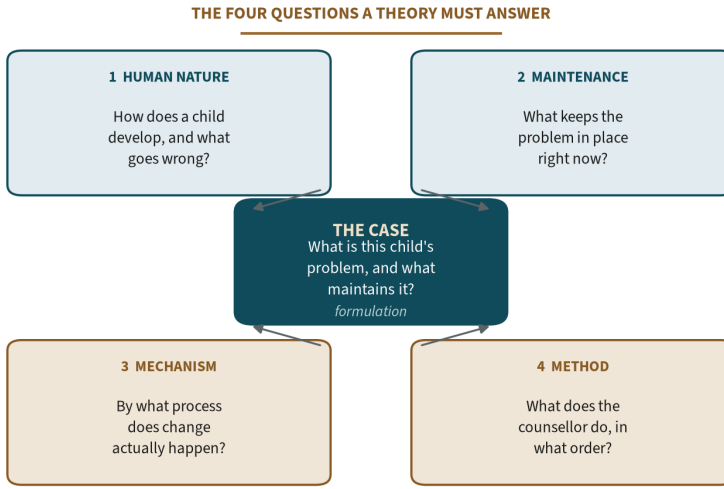


Figure 1.1 The four questions a counselling theory answers, arranged around the case: human nature, maintenance, mechanism, and method. A theory that cannot answer all four is a vocabulary rather than a treatment. *Source:* prepared for this edition.

Question 1 — Human nature and development. What is a child? A bundle of drives requiring channelled expression; an organism whose behaviour is shaped by consequence; a person with an actualising tendency that flourishes under the right conditions; a node in a communication network. Each answer determines what counts as health, and therefore what counts as the target of treatment. This question is where developmental theory enters counselling: a theory that assumes a capacity for reflection cannot be used with a child who has not developed it.

Question 2 — Maintenance. Not "how did this begin?" but "what keeps it going now?" The distinction is the most practically useful in the whole course. Origins are often unrecoverable and rarely modifiable; maintaining factors are present, observable and available to change. A child's school refusal may have begun with a bullying incident nine months ago, but it is maintained this morning by the negative reinforcement of staying home, the mother's anxious checking, the school's informal exemption from attendance, and the erosion of the child's confidence that returning is survivable. Four maintaining factors, four different intervention points, and only one of them is about the bullying.

Question 3 — Mechanism of change. Through what process does the intervention work? Insight and the making conscious of the unconscious; extinction, habituation and new learning; a corrective emotional experience within the therapeutic relationship; restructuring of the family's hierarchy and boundaries;

the construction of an alternative story. Mechanism is the question most often skipped and the one that makes a theory falsifiable: if you claim the mechanism is insight, then a treatment that produces change without insight has refuted something you believed.

Question 4 — Method. What does the counsellor do, in what order, with whom, for how long? Method is downstream of the other three, and a method adopted without them is a technique performed on a child rather than a treatment offered to one.

Two further questions are not part of every theory but should be part of every practitioner's: **what is the evidence** for this approach with this problem in this population, and **for whom** does it hold — which cultures, families, settings and languages was it developed in and tested on? The second question is taken up in week 12 and in CAC. 710; the first in week 14.

Table 1.1 shows how four approaches answer the four questions about the same presenting problem, a nine-year-old's refusal to attend school.

Table 1.1 Four answers to the same case. The differences are not stylistic; each column implies a different person to invite into the room.

Question	Psychodynamic	Behavioural / CBT	Person-centred	Systemic
View of the child	A subject with unconscious conflict and a developing internal world	A learner whose behaviour is shaped by consequences and cognitions	A person with an actualising tendency blocked by conditions of worth	A member of a system whose behaviour carries a function for it
What maintains it	Separation anxiety; unconscious meaning of school; defensive avoidance	Negative reinforcement of staying home; catastrophic cognition; safety behaviours	A relationship in which the child's experience is not accepted	Parental anxiety, enmeshment, unresolved loss, school–family coalition
Mechanism	Insight; containment; making the unconscious conscious; a reliable object	Extinction, new learning, cognitive restructuring, exposure to disconfirmation	Congruence, unconditional positive regard and empathy producing self-acceptance	Changed structure, boundaries and communication; altered function of the symptom
Method	Play, interpretation, the transference relationship, work with parents	Graded exposure, behavioural experiments, parent and teacher contingency work	Non-directive sessions, reflection, no advice, no direction	Family sessions, enactments, reframing, tasks, boundary work

PRACTICE NOTE

Clinical implication. When you read any account of a therapy — a manual, a paper, a training brochure — mark which of the four questions it answers. Marketing material for commercial programmes answers question 4 almost exclusively. That is not a reason to reject the programme; it is a reason not to mistake it for a theory.

Worked example 1.2 — Maintenance versus origin

Task. Amara, aged 11, wets the bed roughly twice a week. It began at age 5, shortly after her father left the home; her mother describes those early years as "the bad time". Current facts: Amara sleeps in her grandmother's room; the grandmother wakes her at 2 a.m. "to be safe" and changes the sheets with loud disapproval; Amara has stopped staying overnight with friends and has declined a school camp; the mother, exhausted, has begun to punish missed mornings by removing phone time. List the *maintaining* factors. Then state, for each, which family member's behaviour would have to change.

Solution. Origin: the separation at age 5 — unrecoverable and, for treatment purposes, largely irrelevant. Maintaining factors: (1) the 2 a.m. waking, which prevents Amara from learning bladder capacity and from waking to her own signals — change required: grandmother; (2) the disapproval at sheet-changing, which adds shame and anxiety, and anxiety increases enuresis risk — change required: grandmother; (3) avoidance of overnight stays, which protects Amara from discovering that the problem is manageable and from peer-normalising — change required: Amara, supported; (4) punishment of missed mornings, which converts a physiological event into a contingency and raises household stress — change required: mother. Four maintaining factors, four change agents, none of them the father who left six years ago. Note what the analysis has done: it has moved the treatment from "talk about Daddy" to four concrete, observable changes in the household's night.

Lecture III — Common factors, specific ingredients, and what the evidence actually shows

Suppose two therapies with incompatible theories of the problem and incompatible mechanisms produce the same outcomes. This happens constantly, and it is the most important empirical fact in the field. Three explanations compete.

The common factors account. Most of the variance in psychotherapy outcome is not attributable to the specific method. Lambert's widely cited decomposition attributed roughly 40 per cent of change to extratherapeutic events in the client's life, 30 per cent to relationship and expectation, 15 per cent to hope and expectancy effects, and 15 per cent to the model-specific technique; Frank and Frank made a similar argument about the healing qualities shared by

all persuasive treatments. Figure 1.2 sets the argument out. The percentages are schematic and contested — they are not measurements — but the direction of the finding has been replicated many times: what the therapist and client do together, and what happens in the client's life, matter more than which school the therapist belongs to.

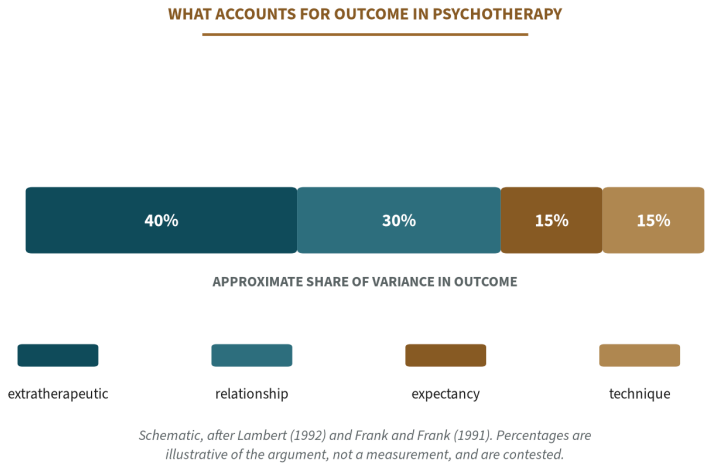


Figure 1.2 A schematic bar showing the approximate share of variance in psychotherapy outcome attributed to extratherapeutic change, relationship and expectation, hope and expectancy, and model-specific technique. Percentages are illustrative of the argument rather than a measurement, and are contested. *Source:* prepared for this edition, after Lambert (1992) and Frank and Frank (1991).

The equivalence account. The "dodo bird verdict" — that all bona fide therapies are roughly equally effective — has been argued from meta-analytic data and disputed from the same data by investigators who find reliable differences for particular disorders. Both sides are partly right, and the practical conclusion is modest: for many common presentations, differences between established therapies are small relative to differences between a competent therapy and no therapy, or between a good alliance and a poor one.

The specificity account. Some treatments do outperform others for some problems, and the clearest examples are in child and adolescent work: behavioural parent training for conduct problems, exposure-based treatment for anxiety, and trauma-focused cognitive-behavioural therapy for post-traumatic stress all have evidence that is stronger than the evidence for undifferentiated supportive counselling. Specificity is real; it is simply narrower than advocates claim.

Three consequences follow for a beginning practitioner, and they are the practical content of this lecture.

First, **the relationship is not the soft part of the work**. Alliance, empathy, agreement on goals and tasks, and the credibility of the treatment to the child and the parent are among the strongest predictors of outcome, and they are trainable. In work with children the alliance is triadic — child, parent, counsellor — and it is common for the adult alliance to be good while the child's is not, which is a serious problem that looks like success.

Second, **allegiance and depth of training matter more than choice of school**. Outcomes are better where the practitioner believes in the method and has been properly trained in it. Eclecticism adopted before competence in any single approach tends to produce drift rather than flexibility, and drift is invisible to the person drifting.

Third, **evidence-based practice is a three-legged stool, not a list**. Sackett and colleagues defined it as the integration of best available research with clinical expertise and with the patient's values and circumstances; Kazdin's distinction between evidence-based *treatments* (manualised methods that have been tested) and evidence-based *practice* (the whole process of deciding what to do with this child) is the version that matters here. A list of approved programmes answers the research leg and ignores the other two. Weisz and Kazdin's collection is the standard reference for what has actually been tested in children and adolescents.

> **PRACTICE NOTE**
> **Clinical implication.** When you read an effectiveness claim, ask three questions before you believe it: what was the comparison condition (waiting list, treatment as usual, or an active alternative)? who was the population (age, setting, comorbidity, culture — and was it children, or adults generalised downwards)? and what was measured (a rating scale completed by the parent, the teacher, the child, or an independent observer — all of which disagree more than you would expect)?

Worked example 1.3 — Reading an effectiveness claim

Task. A paper reports: "In a sample of 62 children aged 8–12 referred to a university clinic for anxiety, the new programme produced significantly greater reduction in parent-rated anxiety than a waitlist control at post-treatment ($p < .001$), maintained at three-month follow-up." Apply the three tests.

Solution. Comparison condition: waitlist. Waitlist controls answer "is this better than nothing for three months?" and almost any credible treatment is; they do not answer "is this better than an existing treatment?" Population: university-clinic referrals, which over-represent families with transport, time and confidence in institutions, and under-represent the children you will actually meet in a school or a district clinic. Measurement: parent-rated anxiety only. Parents are not blind to condition, they have an investment in the treatment having worked, and parent and child ratings of child anxiety correlate modestly at best. Verdict: the claim

supports "probably better than nothing, for anxious children of engaged families, as their parents see it" — which is not nothing, and is not what the abstract implies.

CLINICAL VIGNETTE — FICTITIOUS

Daniel — a fictitious composite. Daniel is nine. He has not attended school for eleven days. The stated reason changes: stomach ache, then a boy who looks at him, then that the work is too hard, then nothing at all. His mother sits with him each morning; his father works nights and describes Daniel as "fine, just soft". The family moved seven months ago. In the first session Daniel draws a house with no door, says "that's not for you to see", and asks twice whether his mother is still in the waiting room.

Reading 1 — attachment and internal experience. The move disrupted a peer network; the father's absence leaves the mother as the sole regulator of anxiety; Daniel's repeated checking of her presence and his doorless house suggest separation anxiety with an inhibited expression of it. The mechanism of change would be the child's experience of a reliable other and the parents' capacity to tolerate his distress. The method would include play, gentle exploration of the separation, and work with the mother's own anxiety.

Reading 2 — behavioural and cognitive. Not attending is negatively reinforced every morning: distress falls when Daniel stays home. The mother's presence is part of the reinforcement schedule. The cognition is catastrophic ("something bad will happen, and I could not cope") and is never tested because avoidance prevents disconfirmation. The mechanism would be extinction of avoidance and disconfirmation of the catastrophic belief. The method would be graded return, parent and teacher contingency work, and behavioural experiments.

Both readings fit the facts. They imply different people to invite into the room, different first sessions, and different things to count as progress. A third reading — systemic — would ask what the symptom is doing for the family, and would notice that Daniel's absence keeps a mother at home and a father's night shifts unchallenged. Daniel returns in weeks 5, 10 and 15 as the course's running case; keep your notes on him.

SKILLS LABORATORY

Practical 1 — formulation under constraint. Four timed steps, alone or in pairs, then debrief.

1. **Five minutes.** Read the vignette once. Write a single sentence: *Daniel's difficulty is maintained by* **, and** *would change it.* No theory names allowed.
2. **Ten minutes.** Write the same case twice, once from an attachment or psychodynamic position and once from a behavioural position. Each account must name a mechanism and one observation that would support it.

3. **Ten minutes.** Compare. Identify one prediction each account makes about what will happen in the next session that the other does not make. This is the test of whether you have two theories or one theory twice.
4. **Five minutes.** Role-play: explain to Daniel's mother, in ordinary language and in under two minutes, what you think is happening and what you propose to do. No jargon. She is sceptical and says, "So you just want him to talk about his feelings?"

Debrief. Which account generated a testable prediction? Which generated a description that no observation could contradict? What did the mother's question expose about your explanation?

ASSESSMENT

Assessed task. A comparative formulation of 1,500 words. You are given a second fictitious case at the end of the teaching week. Write a formulation of it from two specified theoretical positions, state each position's mechanism of change, identify the one observation that would discriminate between them, and recommend which you would act on and why.

Criterion	Weight
Each formulation names a mechanism of change, not only a description	30%
The discriminating observation is genuinely discriminating	25%
Developmental status of the child is used in the argument	20%
The recommendation is justified and its risks acknowledged	15%
APA 7th citation and prose quality	10%

Formative. Submit the one-sentence formulation from step 1 of the practical by the end of teaching week 1. Feedback is returned within five working days and is not graded.

UNIT SYNTHESIS

Synthesis. A counselling theory is a set of answers to four questions — what a child is, what maintains the difficulty, by what mechanism change occurs, and what the counsellor therefore does — and the practitioner's task is not to choose a school but to be able to state, for this child, what they are relying on and how they would know it failed.

READING (APA 7TH)

- Corey, G. (2017). *Theory and practice of counseling and psychotherapy* (10th ed.). Cengage. — **core text**; chapters 1–2 for the comparative frame used throughout this course.
- Frank, J. D., & Frank, J. B. (1991). *Persuasion and healing: A comparative study of psychotherapy* (3rd ed.). Johns Hopkins University Press.
- Kazdin, A. E. (2008). Evidence-based treatment and practice: New opportunities to bridge clinical research and practice, enhance the knowledge base, and improve patient care. *American Psychologist, 63*(3), 146–159.
- Lambert, M. J. (1992). Implications of outcome research for psychotherapy integration. In J. C. Norcross & M. R. Goldfried (Eds.), *Handbook of psychotherapy integration* (pp. 94–129). Basic Books.
- Norcross, J. C., & Lambert, M. J. (2019). Psychotherapy relationships that work III. *Psychotherapy, 56*(4), 503–515.
- Weisz, J. R., & Kazdin, A. E. (Eds.). (2017). *Evidence-based psychotherapies for children and adolescents* (3rd ed.). Guilford Press.

Seminar questions

1. A colleague says, "I don't do theory, I do what works." Reconstruct the theory they are actually using — its view of the child, its account of maintenance, its mechanism, its method — and identify one situation in which it would predictably fail.
2. If most of the variance in outcome lies outside the technique, what is the justification for training practitioners in specific models at all? Argue both sides, then take a position and state what would change it.
3. Take a case you have observed or read about and write four formulations of it, one per column of Table 1.1. Which would you act on, and what would have to be true for you to be wrong?
4. Why is the distinction between origin and maintenance ethically important as well as clinically useful, when the family believes the origin is the point?
5. What would you say to a parent who asks, "Which therapy is the best one?" Answer in the terms of this week, in under a minute.

Next week

Week 2 begins the first lineage: Freud, Anna Freud and the discovery that a child cannot be treated by the methods built for adults, and that play is the child's analytic instrument. Read the account of play as communication in Geldard, Geldard and Foo before the session, and bring one example of a child's play you have witnessed that you would call communicative and one you would not.

2 LECTURE UNIT 2 Teaching week 2 4 notional hours

The Psychodynamic Lineage I: Freud, Anna Freud, and the Child Who Cannot Free-Associate

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Explain** why the classical analytic method could not be applied to children unaltered, and state the two modifications Anna Freud and Melanie Klein each made.
2. **Describe** at least four of the therapeutic powers of play identified in the play-therapy literature, and match each to a mechanism of change.
3. **Analyse** a transcript of a child's play session using the four channels of evidence (content, relationship, symbol, regulation), producing one hypothesis per channel.
4. **Distinguish** interpretation, containment and play-as-communication as three different mechanisms, and say which a given session excerpt illustrates.
5. **State two limits** of the psychodynamic evidence base with children, and what would strengthen it.

Where this week sits

Week 1 gave you four questions. This week begins answering them with the oldest lineage in the field, and with the discovery that matters most for our programme: the child is not a small adult, and a method built for an adult's couch fails in a room with a seven-year-old. Everything later in the course — attachment in week 3, mentalisation in week 4 — grows out of the attempt to take the child's actual mind seriously.

Key concepts

THE UNCONSCIOUS, OPERATIONALLY

Not a place but a claim: that some of what a child does is organised by wishes, fears and memories they cannot report and would deny. In the room: the child who agrees to a plan on Monday and destroys it on Tuesday without knowing why. The claim is testable in a weak but real sense — a formulation built on it should predict repetitions.

DEFENCE

The mind's way of keeping unbearable material at a distance: repression, denial, projection, displacement, reaction formation, and — most visible in children —

regression. In the room: the eleven-year-old who answers every question about her parents with a detailed account of her football team. Not resistance to be broken; a solution to be respected and, slowly, made unnecessary.

PLAY AS COMMUNICATION
The child's equivalent of free association. Play is not a reward for cooperating with talk; it is the medium in which the child thinks aloud. In the room: the difference between watching what a child plays and watching *how* the child plays — who is cast in which role, what is not allowed to happen, where the story always breaks.

CONTAINMENT
The function by which an adult receives a child's unmanageable feeling, holds it without being destroyed by it or retaliating with it, and returns it in a form the child can use. In the room: what you are doing when a child's rage meets your unchanged face and unchanged schedule. Mechanism, not manner.

Lecture I — Why the classical method failed with children

Proposition 1. The classical method required capacities children do not yet have. Psycho-analysis as Freud developed it for adults depends on free association, on the capacity to observe one's own thoughts, on a sustained verbal exchange, and on a motive to understand oneself that is strong enough to survive the pain of understanding. A child of six has none of these in usable form. The child cannot lie down and say whatever comes to mind; what comes to mind arrives as play, drawing, movement and symptom.

Proposition 2. The child also arrives without a request for help. Adults come complaining of their suffering. Children are brought, usually by the person the child's symptom is most organised around. The analyst therefore meets a client who has not asked for treatment and a referrer who has. Anna Freud treated this as the central technical problem of child analysis: the analytic situation must first be made wanted, which means the analyst begins as a provider of something the child values before becoming an interpreter of anything.

Proposition 3. The child's dependence changes the ethics and the technique together. An adult can leave. A child cannot, practically or emotionally, and the analyst is always also an adult authority in a world of adult authorities. Anna Freud's conclusion — that analysis with children cannot be neutral in the way analysis with adults aims to be, and must include an educational and protective dimension — was controversial and remains correct. Klein disagreed about almost everything else but not about the gravity of the asymmetry.

Proposition 4. Two modifications followed, and they define the lineage. Anna Freud's: meet the child where the child is, build the alliance before interpreting, work closely with the parents, and interpret defence before impulse. Klein's: the child's play *is* association and can be interpreted directly, symbol by symbol, in the transference, from the first session. Between those two positions — alliance-first and interpretation-first — the whole of subsequent child psychotherapy has oscillated.

> **PRACTICE NOTE**
> **Clinical implication.** Notice which modification you instinctively prefer and why. Practitioners who cannot tolerate not-knowing drift to interpretation-first and interpret material that is not yet there. Practitioners who cannot tolerate the child's pain drift to alliance-only and never interpret at all. Both are defences; name yours.

Worked example 2.1 — Interpretation or containment?

Task. In a session, Kofi (7) spends twenty minutes building a wall of blocks around a small animal figure, then knocks the wall down, looks at you, and rebuilds it higher. He does this three times. On the fourth attempt he stops, holds the animal, and says nothing. Classify what the following three responses would each be doing: (a) "You keep making sure he's safe, and then it isn't safe enough." (b) Sitting with the silence, matching his stillness, and saying after a while, "It's a lot of work, keeping him safe." (c) Asking, "Who is the animal?"

Solution. (a) is an interpretation: it names the repetition and its meaning (safety that never becomes enough) and offers it to the child's understanding. It is well-timed only if the alliance can carry it; in session three it probably cannot, and an early interpretation is experienced as being seen through, not seen. (b) is containment with a marked mirror: the feeling is received, held, and returned in words that are his but calmer. The mechanism is regulation, not insight. (c) is neither; it is an invitation to symbolise verbally, which a seven-year-old may experience as a demand and which collapses the play into a questionnaire. For this session, (b); the interpretation (a) belongs to a later session when the repetition has been shared many times and named by the child first.

Lecture II — Play as the analytic instrument

If play is association, then the playroom is a text and the counsellor is a reader. Schaefer and Drewes organise the therapeutic powers of play into four groups: communication, emotional healing, social competence and personal strength. Four are worth learning first because each carries a mechanism.

Play communicates what cannot be said. A child's narrative choices — who is powerful, who is helpless, who is absent, what is not allowed to occur — carry

material the child cannot report. The mechanism is expression: material that is enacted is material that can be responded to.

Play regulates affect at a survivable dose. In play the child controls the distance to the frightening thing. The monster is in the box; the box is closed; the child opens it when ready. The mechanism is graduated exposure under the child's own control, which is why play therapy can be effective for anxiety and post-traumatic material without ever becoming a protocol.

Play allows the forbidden in symbolic form. Wishes that would be dangerous to express directly — to destroy the rival, to abandon the parent — can be expressed by a proxy and survive the expression. The mechanism is displacement with safety: the impulse is discharged and observed without consequence.

Play rehearses mastery. The child who was helpless becomes, in play, the one who rescues, decides, repairs. The mechanism is enactment of the counterposition: competence practised where failure costs nothing.

Figure 2.2 sets out the four channels through which the playroom yields evidence, and the discipline that keeps the reading honest.

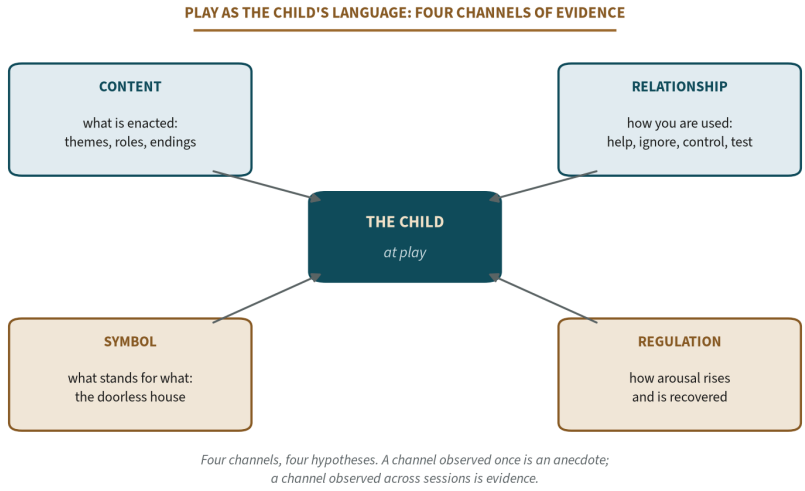


Figure 2.1 Play as the child's language: four channels of evidence — content, relationship, symbol and regulation — each yielding a hypothesis about the child rather than a conclusion. A channel observed once is an anecdote; a channel observed across sessions is evidence. *Source:* prepared for this edition.

SAFEGUARDING AND DUTY TO REPORT

Play therapy with children routinely surfaces disclosures of harm. A child who enacts violence against a doll, or sexualised play beyond developmental expectation, may be communicating abuse. Play content is not in itself evidence and must never

> be over-read; but a disclosure in a session — spoken or enacted and then spoken — engages your safeguarding duty immediately. Follow the procedure of your jurisdiction and placement, record what was said and done in the child's words, and do not interrogate the child to strengthen an account. See CAC. 703 for the full framework; the duty is assumed from this week forward.

Lecture III — The structural and topographic map, and what survives of it

Freud's map of the mind — conscious, preconscious, unconscious; id, ego, superego — is now mostly of historical interest as a model of cognition, and no serious practitioner reads a child's mind through it literally. Three of its claims, however, survive in modern form and are worth keeping.

First, that much of mental life is not available to report. Cognitive science has absorbed this without the hydraulics: implicit memory, automatic appraisal, procedural knowledge of relationships. When a psychodynamic clinician says a child's internal working model is unconscious, the claim is close to what attachment research means by implicit relational knowledge.

Second, that the mind manages conflict by distance. Defence is now studied as emotion regulation strategy, and the developmental sequence of defences maps recognisably onto the developmental sequence of regulatory strategies.

Third, that early relationships become structure. This is the claim that attachment theory operationalised and tested, and it is the bridge to next week.

Figure 2.1 shows the classical map as the lineage states it, with the modern gloss attached. Read it as the lineage's self-description, not as current cognitive science.

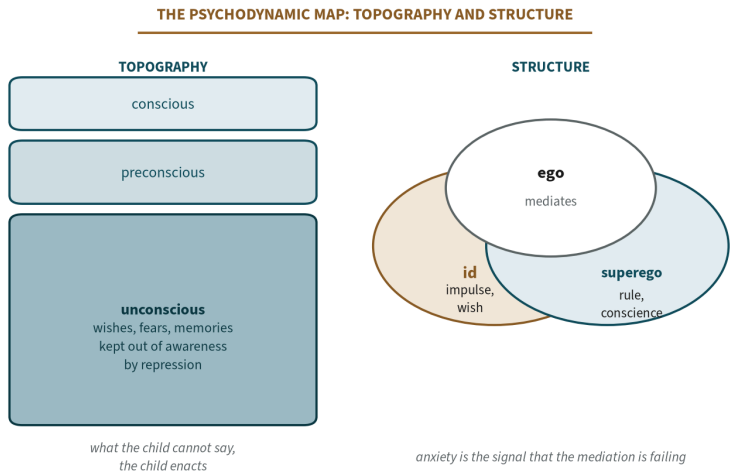


Figure 2.2 The psychodynamic map in two panels: topography as depth layers of conscious, preconscious and unconscious, and structure as the mediating ego between impulse and rule, with anxiety as the signal of failed mediation. Read as the lineage's self-description rather than as current cognitive science. *Source:* prepared for this edition.

PRACTICE NOTE

Clinical implication. The evidence base for psychodynamic therapy with children is thinner than for behavioural and cognitive methods: fewer randomised trials, smaller samples, and outcome measures that privilege the theory. Midgley and colleagues' work on child psychotherapy outcome is the honest state of the art. This does not make the lineage useless; it makes it a lineage whose claims you must hold as hypotheses. The capacities it names — containment, symbolic expression, the repetition of relational patterns — are observable in every session whatever your school, and every school borrows them.

Worked example 2.2 — Four channels, four hypotheses

Task. Read the following excerpt from a fictitious session with Ama (6), referred after her mother's new partner moved in. Ama plays with a family of dolls. She seats the mother doll and the baby doll together, then places the father doll on a shelf facing the wall. A large dog doll enters and knocks over the baby doll. Ama right the baby, puts the dog outside the house, then brings the dog back in and seats it at the table. She looks at you and says, "He's not bad. He's just big." Produce one hypothesis per channel.

Solution. *Content:* the recurring sequence is intrusion, damage, repair, and re-admission — the new partner damages and is nevertheless seated at the table; hypothesis: the child is working out whether the intrusion can be made safe rather

than expelled. *Relationship:* Ama looks at you before naming the dog's goodness; hypothesis: she is testing whether you will agree that the big thing can be good, i.e. whether loyalty to the absent father will be punished here. *Symbol:* the father doll faces the wall — excluded but present, watching; hypothesis: the child holds the father as observer of the new arrangement, and guilt about enjoying the new partner may be part of the problem. *Regulation:* arousal rises at the knock and falls at the re-seating; the play recovers without your help; hypothesis: regulatory capacity is intact and the play is mastery rather than overwhelm. Four hypotheses, none a conclusion; next week's sessions test them.

SKILLS LABORATORY

Practical 2 — the playroom transcript. In pairs. One partner plays a six-minute "session" with any available objects while the other observes and notes only observable events (what is chosen, what happens to it, who is cast where, where the story breaks, where arousal rises and falls). Then the observer produces four hypotheses, one per channel, each marked with the observation it rests on and one observation that would disconfirm it. Swap. Debrief on the discipline: how many of your first instincts survived being written as testable hypotheses?

ASSESSMENT

Assessed task. A play-analysis of 1,000 words on a supplied fictitious session transcript: four channel-hypotheses, the mechanism each implies, and a statement of what you would do in the next session and why.

Criterion	Weight
Each hypothesis is tied to a named observation	35%
Mechanisms distinguished from descriptions	25%
Disconfirming observation offered for each	20%
Next-session plan follows from the hypotheses	10%
APA 7th and prose quality	10%

Formative. Bring one example of play you have witnessed that you would call communicative and one you would not, with a sentence on the difference.

UNIT SYNTHESIS

Synthesis. The psychodynamic lineage's durable contribution is the claim that the child's mind must be met in the child's medium — play, symbol and relationship — and that what cannot be said can still be observed, held and, in time, understood.

READING (APA 7TH)

- A. Freud. (1946). *The psycho-analytical treatment of children* (2nd ed.). International Universities Press.
- Axline, V. M. (1947). *Play therapy: The inner dynamics of childhood*. Houghton Mifflin.
- Geldard, K., Geldard, D., & Foo, R. Y. (2018). *Counselling children: A practical introduction* (5th ed.). SAGE. — **core text**; the play-therapy chapters.
- Klein, M. (1932). *The psycho-analysis of children*. Hogarth.
- Midgley, N., & Kennedy, E. (2011). Psychodynamic psychotherapy for children and adolescents: A critical review of the evidence base. *Journal of Child Psychotherapy, 37*(3), 277–300.
- Schaefer, C. E., & Drewes, A. A. (2011). *The therapeutic powers of play: 20 core agents of change* (2nd ed.). Wiley.
- Winnicott, D. W. (1971). *Playing and reality*. Tavistock.

Seminar questions

1. Anna Freud insisted on building the alliance before interpreting; Klein interpreted from the first session. What does each position assume about the child's capacity to bear being understood?
2. Is containment a mechanism or a manner? Defend your answer with an observation that would distinguish the two.
3. A parent asks why their child "just plays" in sessions. Compose your reply in ordinary language, and identify the mechanism you are claiming.
4. What would count as evidence that symbolic play, rather than the relationship, produced a change?
5. Where is the line between reading a symbol and imposing one? Give an example of an over-reading you have seen or made.

Next week

Week 3 moves from the drive model to the relational one: Bowlby's attachment theory, the Strange Situation, and the secure base. Read the description of the Strange Situation procedure and the four categories before the session; bring one example from your own observation of a child using an adult as a safe haven and one of a child not doing so.

3 LECTURE UNIT 3 Teaching week 3 4 notional hours

The Psychodynamic Lineage II: Attachment, the Strange Situation, and the Secure Base

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Describe** the Strange Situation procedure and the four attachment categories it yields, including what each category's behaviour is organised around.
2. **Explain** the secure-base cycle and identify, in a given case, the point at which the cycle breaks.
3. **Use** the concepts of internal working model, safe haven and secure base to formulate one case, distinguishing them from each other.
4. **Evaluate** two limits of attachment-based practice: the gap between infant categories and later personality, and the risk of using "attachment" as an untestable explanation.
5. **State** why disorganised attachment matters clinically and what it obliges the practitioner to do differently.

Where this week sits

Week 2 ended with the claim that early relationships become structure. This week examines the research programme that tested that claim and made it measurable. Attachment theory is the psychodynamic lineage's most durable gift to practice with children, and also its most over-used word: "attachment" is invoked to explain everything from clinginess to arson. By the end of this week you should be able to use the concept precisely, and to notice when others are not.

Key concepts

ATTACHMENT BEHAVIOUR

Behaviour whose predictable outcome is proximity to a particular person under conditions of fear, fatigue or illness — crying, following, clinging, reaching. It is a behavioural system with a function, not a sentiment. In the room: the child who checks whether mother is in the waiting room is running attachment behaviour; the child who does not check may be running it too, in the deactivating form.

SAFE HAVEN AND SECURE BASE

Two directions of one relationship. The safe haven is who the child goes to when frightened; the secure base is who the child goes *from* when exploring. In the room: a child who has neither will not play, because play requires confidence that the world can be left and returned to.

INTERNAL WORKING MODEL

The child's accumulated, largely implicit expectations of what happens when need is expressed: whether the signal will be answered, and how. In the room: the reason

two children respond identically to your warmth with opposite meanings — one relieved, one suspicious.

SENSITIVITY
The caregiver's capacity to notice the child's signal, read it correctly, and respond promptly and appropriately. Not love, not commitment, not resources: accuracy and timing. In the room: the variable you are actually working on in most parenting-focused work.

Lecture I — From drive to relationship: what Bowlby changed

Proposition 1. Attachment is not a derivative of feeding. The psychoanalytic and learning traditions both derived the child's tie to the mother from the satisfaction of hunger. Bowlby's argument, built on ethology and on the observation of separated children, was that the tie is primary: proximity-seeking is its own system with its own function, the protection of the immature organism. Harlow's work with infant monkeys — preference for the cloth surrogate over the wire one that fed them — made the point unanswerable, though its method would not be permitted today and is cited here as history.

Proposition 2. Separation has an observable sequence. Observation of young children separated from a parent showed protest, then despair, then detachment — and that detachment, the apparent recovery, is the most worrying phase, because it is the child's solution to an unbearable situation and it generalises. Robertson's films of hospitalised children changed practice in hospitals and, eventually, in courts and care systems.

Proposition 3. The Strange Situation made the theory measurable. Ainsworth's procedure — a series of separations and reunions with a stranger in an unfamiliar room — compresses the attachment system into twenty minutes and makes the reunion observable. What is coded is not the separation distress but the *organisation of behaviour at reunion*: how the child resolves the conflict between the wish for comfort and whatever else the relationship has taught.

Proposition 4. Four organisations, not four temperaments. In the secure pattern (B) distress is signalled and comfort accepted, and play resumes. In the avoidant pattern (A) the signal is minimised: the child looks fine and is not, physiologically. In the resistant pattern (C) the signal is maximised and comfort does not settle it. In the disorganised pattern (D), added later by Main and Solomon, there is no coherent strategy: the child approaches and freezes, stills, or shows contradictory behaviour, because the person who would be the solution is also, in some way, the source of alarm. Figure 3.1 places the four on the two dimensions the procedure measures.

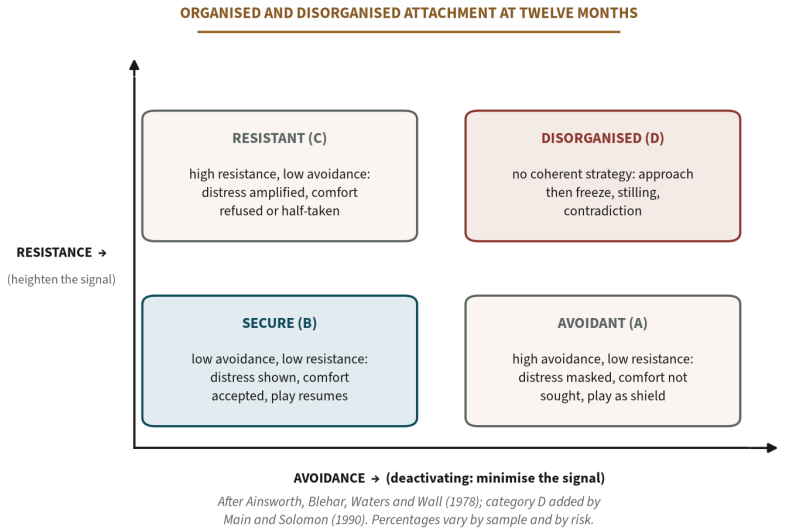


Figure 3.1 Organised and disorganised attachment at twelve months plotted on avoidance and resistance: secure low on both, avoidant high avoidance, resistant high resistance, disorganised without a coherent strategy. After Ainsworth, Blehar, Waters and Wall (1978), with category D added by Main and Solomon (1990). *Source:* prepared for this edition.

PRACTICE NOTE

Clinical implication. A is not "cold", C is not "manipulative", D is not "naughty". Each category is the best organisation the child could build with the caregiver available. When you feel moral judgement rising toward a child's strategy, you have stopped formulating.

Worked example 3.1 — Reading a reunion

Task. A nursery records a reunion on video for a settling-in assessment. Tunde (14 months) continues playing when his mother leaves, does not look up when she returns, and turns his body away while reaching briefly toward her, the reach abandoned. His heart rate, measured in a research context, rose at separation. Classify, and say what the classification implies about the household.

Solution. The pattern is avoidant (A): distress at separation (physiological), minimised signalling at reunion, attention turned to objects, an aborted approach. The implication is not that the mother is unloving but that bids for comfort have, repeatedly, not been answered or have been answered with rejection, so the child has organised around not bidding. Intervention targets the caregiver's response to distress signals, not the child's "independence", which is the strategy and not the problem. Note the trap: to an untrained observer

Tunde looks settled and easy. Avoidant children are frequently described as "good babies".

Lecture II — The secure-base cycle in practice

Figure 3.2 draws the cycle the whole lineage is trying to protect: exploration out, signal when threatened, safe haven, co-regulation, and out again. Three clinical uses follow.

First, as an assessment frame. For any child, ask four questions: who is the safe haven, is the haven actually safe, what happens to the signal on the way to the haven, and what happens to exploration afterwards. A child in care may have a haven on paper and none in fact; a child whose haven responds inconsistently will signal loudly (C); a child whose haven punishes signalling will not signal at all (A); a child whose haven is frightening will show no workable strategy (D).

Second, as a treatment target. Most parenting-focused interventions — including the behavioural parent training of week 5 and the dyadic work of CAC. 728 — can be described as repairs to one segment of the cycle: helping the caregiver notice the signal (sensitivity), tolerate the distress (co-regulation), or survive the reunion (repair after rupture). Naming the segment tells you which sessions to run.

Third, as an account of what the counsellor is. With an avoidant child you will be tested by apparent indifference; with a resistant child by protest that does not settle; with a disorganised child by fear of you. The counsellor's task is to become a predictable segment of the cycle without pretending to replace the caregiver — a temporary secure base whose job is to make the real one usable again.

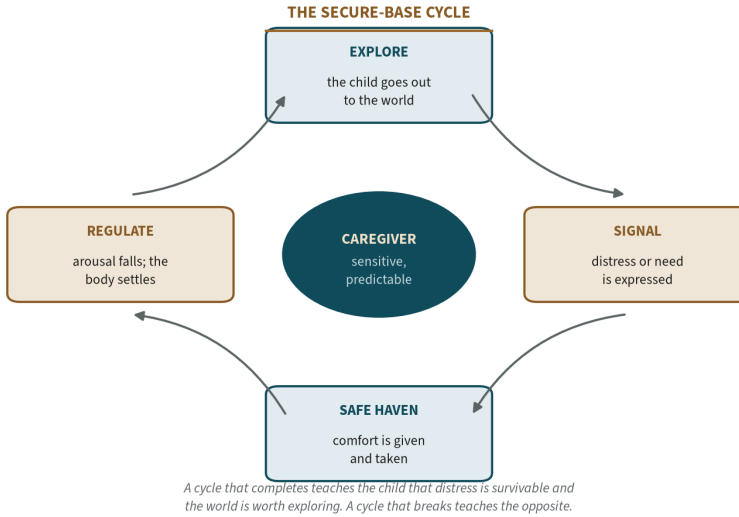


Figure 3.2 The secure-base cycle: exploration, signal, safe haven and regulation, arranged around a sensitive and predictable caregiver; a cycle that completes teaches the child that distress is survivable and the world worth exploring, a cycle that breaks teaches the opposite. *Source:* prepared for this edition.

SAFEGUARDING AND DUTY TO REPORT

Disorganised attachment is strongly associated with caregiving that is frightening, frightened or frighteningly absent, and therefore with maltreatment. Where a child shows fear without solution toward a caregiver — freezing, stilling, approach-avoidance in the same moment — treat it as a safeguarding indicator requiring assessment of the caregiving environment, not merely as a treatment consideration. Follow your jurisdiction's and placement's procedure.

Lecture III — What attachment theory does not say

Three over-extensions are common in practice and in court reports, and you should not repeat them.

The category is not a personality. Infant attachment categories predict group-level outcomes modestly and probabilistically; they are not diagnoses of the child's character, and a B classification at twelve months is not a warranty. The Minnesota longitudinal study shows the predictive power of early care operating largely *through* subsequent care and circumstance: early patterns matter, and later experience keeps mattering.

"Attachment disorder" is not the same thing as insecure attachment. The clinical diagnoses of reactive attachment disorder and disinhibited social engagement disorder describe rare patterns associated with severe early neglect

and institutional care. They are not labels for difficult behaviour in looked-after children generally, and their casual use has real consequences for the child labelled.

The mother is not the only attachment figure, and attachment is not the only relationship that matters. Children build hierarchies of attachment figures including fathers, grandparents and older siblings, and their development depends on many relationships besides attachment ones — teaching, friendship, mentorship. A formulation that reduces a child's difficulties to "attachment" has usually stopped looking.

> **PRACTICE NOTE**
> **Clinical implication.** When a report states that a child has an "attachment problem", ask three questions: on what observation, using what procedure, against what comparison? If the answer is "the child clings to me", the report has described a behaviour and named a theory over it.

Worked example 3.2 — Where the cycle breaks

Task. Zainab (4) is in foster care, third placement in fourteen months. In the current placement she is described as "indiscriminately affectionate": she sits on the foster carer's lap within minutes of meeting visitors, calls strangers "mummy", and shows no preference for the carer at reunion after nursery. Name the segment of the cycle that is broken, the hypothesis it supports, and the first thing you would do.

Solution. The broken segment is the specificity of the safe haven: the signal is dispatched to anyone, which means it has never been reliably answered by anyone in particular. The hypothesis is not "too much attachment" but the absence of an organised attachment relationship — the pattern associated with disinhibited social engagement, and a pointer to the quality of early care rather than to the current carer's adequacy. The first thing to do is not to work with Zainab's sociability but to stabilise the placement and work with the carer on being predictably, specifically available: the cycle cannot be repaired with a different adult every quarter. Note what the formulation prevents — a misreading of the child's friendliness as health, and a misreading of the carer as unloved and therefore failing.

> **SKILLS LABORATORY**
> **Practical 3 — the cycle audit.** Using a case you know professionally or a supplied vignette, draw the secure-base cycle for the child: name the safe haven(s), mark where the signal goes, what happens to it on the way, what the haven does with it, and what happens to exploration afterwards. Mark the break. Then write one sentence of intervention aimed only at the broken segment. Compare with a partner: did you both mark the same break? If not, what observation settles it?

ASSESSMENT

Assessed task. A cycle audit and formulation of 1,200 words on a supplied case: the four assessment questions answered from the material, the broken segment named, an intervention aimed at that segment, and a statement of what the formulation does not explain.

Criterion	Weight
The four assessment questions answered from observed material	30%
Broken segment identified and justified	25%
Intervention matched to the segment	20%
Limits of the formulation stated	15%
APA 7th and prose quality	10%

Formative. Submit one example of a child using an adult as a safe haven and one of a child not doing so, each with the observation it rests on.

UNIT SYNTHESIS

Synthesis. Attachment theory gives practice a measurable relational grammar — signal, haven, base, model — and its discipline is as important as its content: a category is an observation under a procedure, never a personality, and "attachment" explains less than it is asked to explain.

READING (APA 7TH)

- Ainsworth, M. D. S., Blehar, M. C., Waters, E., & Wall, S. N. (1978). *Patterns of attachment: A psychological study of the strange situation*. Lawrence Erlbaum.
- Bowlby, J. (1982). *Attachment and loss: Vol. 1. Attachment* (2nd ed.). Basic Books. (Original work published 1969)
- Cassidy, J., & Shaver, P. R. (Eds.). (2016). *Handbook of attachment: Theory, research, and clinical applications* (3rd ed.). Guilford Press. — **core text**; the clinical applications section.
- Holmes, J. (2014). *John Bowlby and attachment theory* (2nd ed.). Routledge.
- Main, M., & Solomon, J. (1990). Procedures for identifying infants as disorganized/disoriented during the Ainsworth Strange Situation. In M. T. Greenberg, D. Cicchetti, & E. M. Cummings (Eds.), *Attachment in the preschool years* (pp. 121–160). University of Chicago Press.
- Sroufe, L. A., Egeland, B., Carlson, E. A., & Collins, W. A. (2005). *The development of the person: The Minnesota study of risk and adaptation from birth to adulthood*. Guilford Press.
- Zeanah, C. H., & Gleason, M. M. (2015). Annual research review: Attachment disorders in early childhood — clinical presentation, causes, correlates, and treatment. *Journal of Child Psychology and Psychiatry, 56*(3), 207–222.

Seminar questions

1. Why is the reunion, rather than the separation, the coded event in the Strange Situation? What does that choice assume about what attachment is?
2. An avoidant child is described by a teacher as "independent and easy". Rewrite the description in the lineage's terms and state what the rewrite changes about how the child should be taught.
3. Under what conditions, if any, should a counsellor become a child's attachment figure? Argue against your own answer.
4. A court report states that a child's difficult behaviour "is caused by disorganised attachment". What is missing from the sentence, and how would you test it?
5. What does attachment theory owe to the psychodynamic lineage, and what did it have to abandon from that lineage to become testable?

Next week

Week 4 completes the lineage with the concept that connects attachment to practice: mentalisation — the capacity to treat oneself and others as beings with minds, and the caregiver function that builds it. Read the description of marked mirroring before the session and bring one example of an adult responding to a child's feeling in a way that made it worse.

4 LECTURE UNIT 4 Teaching week 4 4 notional hours

The Psychodynamic Lineage III: Mentalisation and the Mind That Holds Minds

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Define** mentalisation and distinguish it from empathy, theory of mind and insight.
2. **Describe** the three pre-mentalising modes — psychic equivalence, pretend mode and teleological stance — and identify each in a transcript.
3. **Explain** marked mirroring and state what makes a mirror "marked", using an example.
4. **Apply** the mentalising loop to a session excerpt, locating the point at which the loop broke and whose mind it broke in.
5. **Use** one measure of parental reflective functioning to structure an assessment interview, stating its limits.

Where this week sits

Weeks 2 to 4 are one argument. Week 2: the child's mind must be met in the child's medium. Week 3: the child's expectations of minds are built in early relationships and are measurable. Week 4: the capacity that those relationships build is the capacity to think about minds at all — your client's, yours, and eventually the child's own. Mentalisation is the mechanism the psychodynamic lineage proposes for its own effectiveness, and it is the bridge by which the lineage joins modern developmental science.

Key concepts

MENTALISING
The imaginative activity of perceiving and interpreting behaviour in terms of mental states — wants, feelings, beliefs, intentions — in oneself and in others. Not empathy (feeling with), not theory of mind (the developmental capacity tested in false-belief tasks), not insight (understanding one's own history). In the room: the difference between "he hit me because he is bad" and "he hit me because he thought I was going to leave him".

MARKED MIRRORING
The caregiver's response that shows the child's feeling accurately but not identically — the face says *your* fear, slightly stylised, contained, and survivable, not the adult's own fear. In the room: the tone you use when a child screams — matching enough to be recognised, modulated enough to be bearable. Unmarked mirroring (the adult's real alarm) teaches the child that the feeling is as dangerous as it feels.

REFLECTIVE FUNCTION
The measurable individual difference in the capacity to mentalise, assessed by coding how a person talks about minds — in the Adult Attachment Interview for adults, in the Parent Development Interview for parents about their child. In the room: what you are listening for when a parent says "he knows exactly how to wind me up" (teleological, no mind) versus "I think he provokes me because he's waiting for me to prove I'll stay" (a mind imagining a mind).

PRE-MENTALISING MODES
The ordinary, developmental, and regressed ways of relating to inner reality before or instead of mentalising: **psychic equivalence** (inner is outer; the thought is the thing, so the fear is a fact); **pretend mode** (inner is severed from outer; talk about feelings that changes nothing); **teleological** (only physical outcomes count; only what you *did* proves what you feel). In the room: the three ways a session stalls.

Lecture I — What mentalisation is and is not

Proposition 1. Minds are opaque, and everything social rests on an inference. You cannot observe a child's fear; you observe a face, a posture, a refusal, and you infer. Mentalising is the inference run deliberately, with the knowledge that it is an inference. The non-mentalising error is not the inference but the certainty.

Proposition 2. The capacity is developmental and state-dependent. Children acquire mental-state understanding in a predictable sequence through the preschool years, and the capacity is fragile under arousal: a child — or an adult — who can mentalise calmly cannot do so in panic. Most ruptures in sessions with children are not failures of skill but collapses of mentalising under threat, in the child or in you.

Proposition 3. Mentalising is what the caregiver builds, and play is where it is built. When a caregiver treats the child as a being with a mind — naming, wondering, getting it wrong and correcting — the child acquires the same stance toward themselves. Fonagy and colleagues' proposal is that the child's sense of self as a thinking being is internalised marked mirroring: the child finds their own mind in the caregiver's face, stylised and survivable.

Proposition 4. Psychopathology can be read as a collapse into pre-mentalising modes. In psychic equivalence the anxious child's prediction is a fact and reassurance is a lie; in pretend mode the adolescent's therapy talk is fluent and weightless; in the teleological stance the neglected child reads love only in what is done and bought, and tests you with demands that cannot be satisfied because they ask for proof of a mind. Figure 4.1 draws the loop whose failure produces these states.

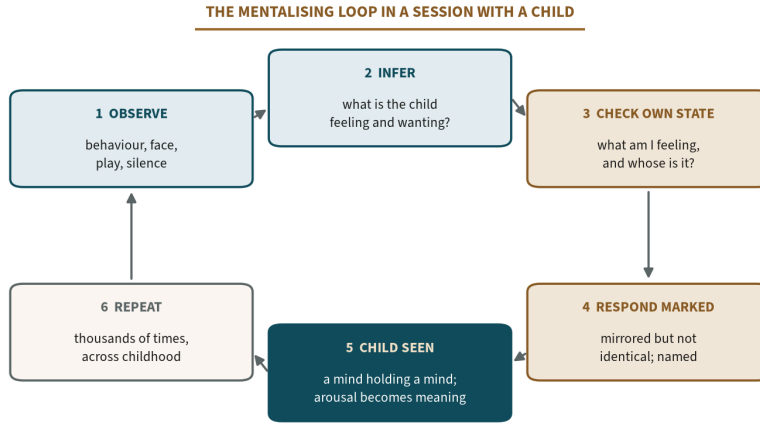


Figure 4.1 The mentalising loop in a session with a child: observe behaviour, infer the child's mental state, check one's own state, respond marked and contingent, so that the child experiences a mind holding a mind and arousal becomes meaning; the loop repeats across childhood. *Source:* prepared for this edition.

PRACTICE NOTE

Clinical implication. When a session stops working, ask first which mode you are both in. Interpreting to a child in psychic equivalence escalates: your interpretation is experienced as an external fact to be fought. The repair is regulation first, mentalising second, interpretation last.

Worked example 4.1 — Naming the mode

Task. Classify each utterance and say what the practitioner's next move should be. (a) Chidi (8), after a nightmare: "The man is in the house. You didn't lock the door properly." (b) Bisi (15), in week six of counselling: "Yeah, I see how my avoidance maintains the anxiety, it's the negative reinforcement cycle," delivered flatly, with no change in any behaviour. (c) Emeka (10), to a foster carer: "If you loved me you would have bought the phone like I asked."

Solution. (a) Psychic equivalence: the dream-content is reality; reassurance ("there is no man") will be heard as dishonesty or stupidity. Next move: take the fear seriously as an experience without confirming the fact — "in the dream he was absolutely in the house; that is what fear does, it makes what it fears real" — then regulate, then explore. (b) Pretend mode: the vocabulary is present and the mind is absent; the therapy has become a seminar. Next move: stop teaching the model and ask what happened this week when the cycle actually ran; bring the discussion back to a specific, felt moment. (c) Teleological: love is only what is

done; the demand is a test of mind, and satisfying it proves nothing because only the object was offered. Next move: do not buy the phone and do not refuse it coldly; name what is being asked for — "you want to know I would do things for you; the phone can't answer that, but I can tell you what I notice about you" — and survive the disappointment without retaliation.

Lecture II — The loop in the room, and where it breaks

Figure 4.1 is a description of ordinary good care, not a technique. Six steps, run thousands of times: observe; infer the child's state; check your own state (whose fear is this?); respond marked and contingent; the child experiences being seen by a mind; repeat. Four failure points are worth memorising because each has a different repair.

Failure at observation. You did not see it — the micro-turn away, the change in play. Repair is ordinary attention and, where possible, video or supervision: practitioners systematically overestimate what they noticed.

Failure at inference. You saw it and read it wrong, usually by reading your own state into the child. Repair is the habit of offering the inference as a question and being willing to be corrected by a nine-year-old, which is one of the most mentalising things an adult can do.

Failure at self-check. Your own alarm or anger enters the response unmarked, and the child meets your state instead of their own. This is the failure that most often ends placements and therapeutic relationships. Repair is supervision and the discipline of naming your state before you act on it.

Failure at marking. The response is accurate but unmodulated — a mirror without a frame — or modulated into falseness, which children detect instantly. Repair is practice: the marked response is a skill with a feel to it, learned by being marked oneself.

> **SAFEGUARDING AND DUTY TO REPORT**
> Mentalising failure is a safeguarding concept as well as a clinical one. Caregiving that is frightening, frightened or persistently unmarked — the carer whose face shows their own terror or rage in response to the child's distress — is both a cause of disorganised attachment and an indicator of risk in the home. Where you observe it, it is reportable material under your safeguarding procedure, not merely a formulation.

Lecture III — Measuring reflective function, and using it without abusing it

Reflective function is coded from interview: the Parent Development Interview asks parents to describe their child and their relationship, and the coding counts evidence that the parent treats the child as a mind — recognising opacity, wondering about states, tracking change, acknowledging the limits of their own

knowledge. Parents high in reflective function have, on average, children with better regulation and fewer difficulties; the association holds over and above income and education, and interventions that raise parental reflective function show effects on parenting and on child outcomes in several trials.

Two cautions govern its use in practice. First, the measures are research instruments requiring training; a clinician who "assesses reflective function" by impression is not measuring anything. Second, low reflective function is not moral failure and not a verdict on love: it is often the residue of a childhood in which nobody mentalised the parent, and it is treatable. The clinical use of the concept is not to sort parents but to decide what to do: with a parent in teleological mode, behavioural work and concrete tasks first, mentalising later; with a parent able to wonder, formulation work can begin immediately.

Mentalisation-based work with adolescents — including the adaptations for self-harm and for borderline presentation in adolescence — applies the same loop to the therapeutic relationship: the therapist's job is to keep mentalising when the adolescent cannot, to mark their own reactions, and to treat every rupture as material about minds rather than as misbehaviour. That stance, more than any technique, is what the lineage hands to the rest of this course.

> **PRACTICE NOTE**
> **Clinical implication.** Adopt one habit from this week and keep it for the rest of the programme: before you respond to a child's strongest feeling, name silently (1) what you think the child is feeling, (2) what you are feeling, and (3) one alternative reading of the child's state. Three seconds. It is the cheapest intervention in the course.

Worked example 4.2 — The rupture as material

Task. In session eight, Funke (12), who has begun to trust you, arrives and announces that the sessions are stupid and she is not coming back. Last session she disclosed something shameful and you responded well, or so you thought. Using the loop, formulate the rupture and plan your response.

Solution. Observe: the announcement follows the disclosure; the timing is the datum. Infer: the shame of having been seen has reorganised as contempt for the seer; the prediction inside her is that you now think less of her, and leaving first converts rejection into choice. Check your own state: you feel stung and, underneath, defensive — the pull to point out her progress, which would be your state, not hers. Respond marked: neither collapse ("of course you can stop") nor defence ("after everything we've done"); instead, naming the sequence as a hypothesis about minds — "last week you told me something hard, and today the sessions are stupid; I wonder if being seen here has started to feel dangerous." The loop's repair is the demonstration that your mind survived her feeling and still

holds an image of her that includes last week's courage. What you must not do is treat the announcement as a decision to be negotiated; it is not a decision, it is a mental state.

SKILLS LABORATORY

Practical 4 — the three-second habit and the marked response. In triads. One plays a child delivering a high-arousal statement (supplied cards: "I hate you", "You're just like my mum", "Nothing works, nothing ever works", "It's not a big deal, stop looking at me like that"). The responder has three seconds to name silently the child's state, their own state, and one alternative reading, then delivers a marked response of one sentence. The third member codes: was the response marked (accurate but modulated), unmarked (the responder's own state), or false (modulated into nothing)? Rotate. Debrief: which statements pulled you into which failure?

ASSESSMENT

Assessed task. A rupture analysis of 1,000 words on a supplied transcript: locate the break in the loop, name the mode each party is in, write the marked response you would give, and state what you would watch for in the next session to know whether the repair worked.

Criterion	Weight
Break located with the observation it rests on	30%
Modes named for both parties	25%
Marked response is marked (accurate, modulated, contingent)	25%
Repair criterion specified for the next session	10%
APA 7th and prose quality	10%

Formative. Submit one example of an adult responding to a child's feeling in a way that made it worse, analysed in terms of marking.

UNIT SYNTHESIS

Synthesis. Mentalisation is the psychodynamic lineage's account of its own mechanism — a mind holding a mind until the child can hold their own — and its practical gift is a discipline: under arousal, regulate first, mentalise second, interpret last.

READING (APA 7TH)

- Allen, J. G., Fonagy, P., & Bateman, A. W. (2008). *Mentalizing in clinical practice*. American Psychiatric Publishing.

- Bateman, A., & Fonagy, P. (Eds.). (2019). *Handbook of mentalizing in mental health practice* (2nd ed.). American Psychiatric Association Publishing. — **core text**; Parts I and II.
- Ensink, K., Mayes, L. C., & Slade, A. (2015). Parental reflective functioning: Conceptualization, assessment, and clinical applications. *Attachment & Human Development, 17*(5), 437–444.
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- Sharp, C., & Fonagy, P. (2008). The parent's capacity to treat the child as a psychological agent: Constructs, measures and implications for developmental psychopathology. *Social Development, 17*(3), 737–754.
- Slade, A. (2005). Parental reflective functioning: An introduction. *Attachment & Human Development, 7*(3), 269–281.

Seminar questions

1. Distinguish mentalising from empathy with an example in which the two come apart, and say which the child needs in a crisis.
2. Why is unmarked mirroring harmful? Give the mechanism in the lineage's own terms and in regulatory terms.
3. A parent says, "He does it to annoy me." Rewrite the sentence at three levels of reflective function and say what each version implies for treatment.
4. Under what arousal conditions does your own mentalising fail, and what is your personal repair? Answer from observation, not aspiration.
5. Is mentalisation-based practice a theory or a stance? What turns on the answer?

Next week

Week 5 changes lineage: from minds to contingencies. Learning theory, functional analysis and the behavioural account of the child's problem. Read the description of the ABC (antecedent–behaviour–consequence) sequence and bring one example of a child's behaviour that is maintained by the very response that punishes it.

5 LECTURE UNIT 5 Teaching week 5 4 notional hours

The Behavioural Lineage I: Learning Theory, Functional Analysis, and the Loop That Maintains

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **State** the difference between respondent and operant learning, and give one child example of each.
2. **Complete a functional analysis** (ABC) from a supplied narrative, identifying the reinforcer that maintains the behaviour.
3. **Predict** the extinction burst and explain, in parent-friendly language, why behaviour worsens before it improves.
4. **Design** a simple contingency change for one maintaining factor, specifying what will be measured and for how long.
5. **State** the ethical limits on behavioural methods with children, including why aversive procedures are excluded from this programme.

Where this week sits

Weeks 2 to 4 asked what goes on inside the child. This week asks what goes on around the child, and the change of question is the whole of the behavioural revolution: not "what does the behaviour mean?" but "what does the behaviour accomplish?" The two questions are not enemies. A formulation that ignores meaning is crude; a formulation that ignores consequence is usually ineffective. Daniel, our school-refusing nine-year-old from week 1, returns this week as a behavioural case, and you will see that the behavioural reading is not a denial of his anxiety but a plan for his mornings.

Key concepts

REINFORCEMENT

Anything that follows a behaviour and makes it more likely. Not a reward, not a treat, not something the adult intends as positive: the child's experience defines it. In the room: the attention a teacher gives to a child who calls out is reinforcement if calling out increases, however much the teacher intends it as a reprimand.

NEGATIVE REINFORCEMENT

The removal of something aversive, which strengthens the behaviour that removed it. The most powerful and least noticed force in child casework. In the room: staying home removes dread, so staying home strengthens; the parent's giving in removes the child's distress and the parent's own, so giving in strengthens. Two loops, one symptom.

FUNCTIONAL ANALYSIS (ABC)

The discipline of writing down antecedent, behaviour and consequence, in order, repeatedly, until the pattern that maintains the behaviour is visible. In the room:

the chart you complete before you propose any change. A behaviour analysed is half treated; a behaviour assumed about is usually mistreated.

EXTINCTION BURST
The temporary increase in a behaviour when its reinforcer is first withdrawn — bigger, louder, longer — followed by decline if the withdrawal holds. In the room: the fortnight in which a programme is abandoned, and the child learns that escalation works. Figure 5.2 is the most practically important graph in the course.

Lecture I — Two kinds of learning, and why the difference matters clinically

Proposition 1. Respondent learning pairs a neutral stimulus with an unconditioned one until the neutral stimulus alone provokes the response. The child bitten by a dog flinches at dogs; the child who was humiliated in a classroom feels dread at classroom doors. The response is not chosen and cannot be argued with. The clinical technology that follows is graduated exposure with prevention of avoidance: the conditioned response declines when the stimulus is met without the unconditioned consequence and without escape.

Proposition 2. Operant learning selects behaviour by its consequences. What is followed by reinforcement recurs; what is followed by nothing or by punishment declines — the latter with side effects (avoidance of the person who punishes, concealment, aggression) that make punishment a poor clinical instrument and, in aversive form, an excluded one.

Proposition 3. The same topography of behaviour can serve different functions in different children. Two children both "have tantrums in the supermarket". For one, the tantrum obtains sweets (positive reinforcement by access); for the other, it ends the shopping trip (negative reinforcement by escape). The identical behaviour requires opposite interventions. This is why functional analysis precedes intervention, and why copying another practitioner's plan for "tantrums" is not practice.

Proposition 4. Social learning completes the picture: children learn by watching. Bandura's demonstration that children reproduce modelled aggression — and, more usefully, modelled coping — put the adult's behaviour inside the model. Every parent, teacher and counsellor is a model whether they intend it or not, which is why behavioural work with children is usually work with the adults around them.

PRACTICE NOTE

Clinical implication. Before any plan, ask of the behaviour: what does it obtain, and what does it escape? If you cannot answer both from observation, you are not yet ready to intervene; you are ready to observe.

Worked example 5.1 — Two functions, one topography

Task. In a Year 3 classroom, Chinedu shouts out during instruction roughly eight times an hour. The teacher's current response is a verbal reprimand each time, delivered at the whiteboard in front of the class. Over three mornings you record: on 6 of 8 occasions the reprimand is followed by peers looking at or answering Chinedu; on 2 of 8 the teacher sends him to the corridor, where he draws quietly and appears calm. Identify the candidate functions and the intervention each implies.

Solution. Candidate function 1: peer attention (positive reinforcement) — supported by the 6 occasions; the teacher's reprimand is the delivery mechanism for the peer audience. Candidate function 2: escape from instruction (negative reinforcement) — supported by the 2 occasions and by his calm in the corridor, which is reinforcing in itself. The two functions require different plans: for attention, deliver attention contingently on raised-hand behaviour and withdraw the audience from shouting (brief, neutral, no reprimand-as-performance); for escape, make the corridor unavailable as an escape route and teach a legitimate way to request a break. Treating only the attention function would leave escape intact and the behaviour would migrate. Note the trap in the teacher's current plan: the reprimand supplies both functions, which is why the behaviour is stable at eight an hour.

Lecture II — Functional analysis as clinical discipline

Figure 5.1 draws the sequence and the loop. Three rules keep the analysis honest.

Rule 1: describe, do not interpret. "Refusal" is not a behaviour; "puts head on desk and does not respond to two prompts" is. Interpretation ("attention-seeking", "manipulative") ends observation and begins prejudice.

Rule 2: the consequence that matters is the one that reliably follows, not the one the adult intends. Intent is irrelevant to selection. Write what happened.

Rule 3: analyse in the setting where the behaviour lives. A functional analysis conducted in an office about a classroom behaviour is a story about a classroom behaviour. Wherever possible, observe, or obtain records from those present: frequency counts, time samples, or a simple ABC log kept for five school days.

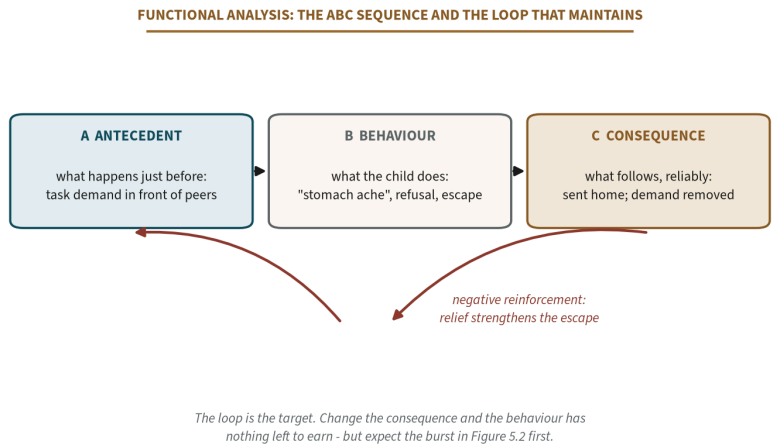


Figure 5.1 Functional analysis as an ABC sequence with the maintenance loop: antecedent, behaviour, consequence, and the negative reinforcement that returns the consequence to the antecedent as a strengthened expectation. The loop is the target of intervention. *Source:* prepared for this edition.

SAFEGUARDING AND DUTY TO REPORT

Behavioural methods carry a history that the profession must own: aversive procedures were used on children, including disabled children, and caused harm. This programme excludes them absolutely. Reinforcement-based plans, extinction of reinforcement for harmful behaviour, and teaching of replacement skills are the permissible instruments; any plan whose mechanism is pain, fear or humiliation is not a treatment and is reportable professional misconduct. Where a child's behaviour is dangerous, safety planning precedes functional analysis, not the reverse.

Lecture III — Changing the consequence, and surviving the burst

If the loop is the target, the intervention is a change in what follows the behaviour, plus teaching of a behaviour that earns the same outcome legitimately. Four design principles follow.

Match the replacement behaviour to the function. A child who shouts out for attention must be able to obtain attention by raising a hand, quickly and reliably, or the plan will fail: extinction without replacement is simply removal.

Make the new contingency immediate and certain. Children discount delay steeply; a token at Friday's assembly cannot compete with attention now. Early in a plan, reinforce every occurrence; thin the schedule only after the behaviour is established.

Expect and plan for the burst. When reinforcement stops, the behaviour escalates before it declines — Figure 5.2. The burst is not evidence of failure;

it is evidence that the reinforcer has been correctly identified. What destroys programmes is adult inconsistency during the burst: reinforcing escalation once teaches the child that escalation works, and produces a behaviour more resistant than the original.

Measure. A plan without a count is an opinion. Frequency, duration or interval recording, kept simply and kept daily, tells you by week two whether the loop has changed. Where measurement is impossible, the plan is not yet ready.

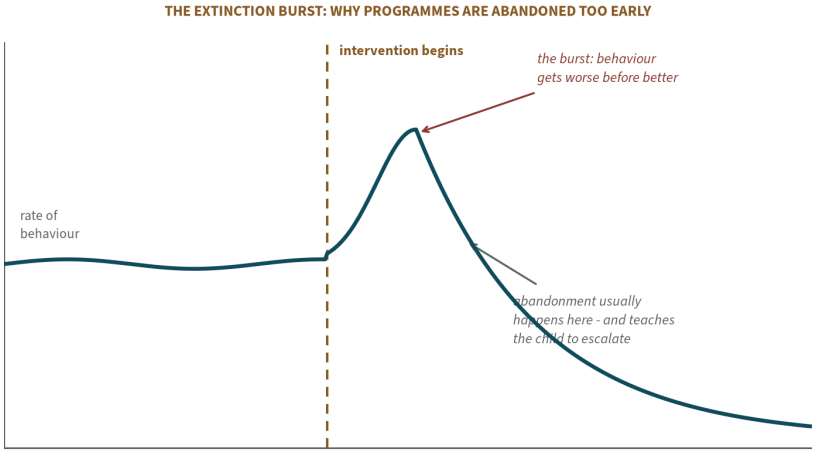


Figure 5.2 The extinction burst: behaviour rate across sessions, with intervention beginning at session eight, a temporary peak as reinforcement is withdrawn, then decline. The annotation marks the point at which programmes are usually abandoned, which teaches the child that escalation works. *Source:* prepared for this edition.

PRACTICE NOTE

Clinical implication. When you present a behavioural plan to parents, present the burst first. A parent who knows that Tuesday and Wednesday will be worse and that this means the plan is working will hold the line; a parent who meets the burst unprepared will abandon the plan and blame the child.

Worked example 5.2 — Daniel's mornings, behaviourally

Task. Return to Daniel (week 1): eleven days of non-attendance, mother sitting with him each morning, distress falling when he stays home. Write the ABC, name the reinforcer, and specify the first two weeks of a plan including what is measured.

Solution. A: morning routine reaches the point of leaving; mother present and available; father absent. B: complaint, distress, refusal. C: stays home; distress falls within the hour; mother's anxious attention is concentrated on him; school demand removed. Reinforcer: negative reinforcement (escape from

school-related dread) plus, probably, positive reinforcement (mother's undivided presence). Plan, weeks 1–2: (1) reinstate attendance in graduated form — first hour only, with a named adult to meet him at the gate — so that attendance is achievable and therefore reinforceable; (2) change the morning consequence: mother's attention is given warmly and abundantly at breakfast and at reunion, and is brief, neutral and boring during refusal; (3) school agrees a predictable, non-negotiable welcome and a predictable, non-punitive routine for the first hour; (4) measure: minutes attended per day, recorded by the school, plus a daily one-line parent log of morning duration. Expect a burst in days 3–6: longer protests, possibly somatic complaints. Brief the mother on it in week 1, in writing. What the plan does not yet address is Daniel's catastrophic cognition — that is week 6's contribution, and the two plans will be combined in week 13.

SKILLS LABORATORY

Practical 5 — the five-day ABC. Obtain (or, if you are not in placement, use the supplied narrative) a five-day record of one child behaviour. Complete an ABC chart for every occurrence. Then write: the behaviour in observable terms; the two candidate functions with the counts that support each; the reinforcer you judge operative; and one sentence of parent-facing explanation of the burst. Swap with a partner and critique: is any cell interpretive rather than descriptive?

ASSESSMENT

Assessed task. A functional analysis and contingency plan of 1,200 words on a supplied case: observable behaviour definition, ABC summary table, function hypothesis with supporting counts, replacement behaviour matched to function, measurement plan, and a parent-facing paragraph that explains the burst.

Criterion	Weight
Behaviour defined observably; no interpretation in the ABC	25%
Function hypothesis supported by the recorded consequences	25%
Replacement behaviour matched to function	20%
Measurement plan feasible in the setting described	15%
Parent-facing paragraph accurate and jargon-free	15%

Formative. Submit your one-sentence statement of the reinforcer for a case of your choosing (fictitious or fully anonymised) by the end of the week.

UNIT SYNTHESIS

Synthesis. Behaviour is selected by its consequences, and the practitioner's first task is to discover what the behaviour accomplishes for this child in this setting;

intervention then changes the consequence, teaches a behaviour that earns the same outcome, and survives the burst.

READING (APA 7TH)

- Bandura, A. (1977). *Social learning theory*. Prentice-Hall.
- Forehand, R., & McMahon, R. J. (1981). *Helping the noncompliant child: A parent-based approach to child behavior*. Guilford Press.
- Patterson, G. R. (1982). *Coercive family process*. Castalia Publishing.
- Skinner, B. F. (1953). *Science and human behavior*. Macmillan.
- Weisz, J. R., & Kazdin, A. E. (Eds.). (2017). *Evidence-based psychotherapies for children and adolescents* (3rd ed.). Guilford Press. — **core text**; the behavioural parent training and exposure chapters.
- Wolpe, J. (1958). *Psychotherapy by reciprocal inhibition*. Stanford University Press.

Seminar questions

1. A teacher says, "I ignore the shouting out and it's getting worse, so ignoring doesn't work." Reconstruct what is actually happening, and what you would change.
2. Why is "attention-seeking" a poor description and a good hypothesis? Reformulate it in the week's terms.
3. Under what conditions, if any, is extinction alone (without a replacement behaviour) defensible with a child?
4. What does the behavioural lineage owe to the observation that parents are part of the contingency schedule, and what does that imply for consent?
5. Where is the boundary between a reinforcement plan and a bribe? Answer in mechanism, not in tone of voice.

Next week

Week 6 adds the cognitive layer: what the child thinks between the situation and the behaviour, hot thoughts, thinking traps, and the behavioural experiment as the engine of change. Read the description of a thought record adapted for children before the session, and bring one example of a child's hot thought you have heard verbatim.

6 LECTURE UNIT 6 Teaching week 6 4 notional hours

The Behavioural Lineage II: Cognitive Therapy with Children and Adolescents

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Draw and explain** the cognitive model (situation–thought–feeling–behaviour) using a child's own example.
2. **Identify** a hot thought and the thinking trap it instantiates, from a verbatim quotation.
3. **Construct** a behavioural experiment with a child that tests a hot thought, specifying the prediction, the test and what would count as disconfirmation.
4. **Adapt** cognitive methods to developmental level, stating what a child of six, ten and fifteen can and cannot yet do.
5. **Evaluate** the evidence for CBT with child anxiety and adolescent depression, naming where it is strong and where it is thin.

Where this week sits

Week 5 located the loop outside the child. This week locates a second loop inside: what the child thinks between the situation and the response, and the way feeling confirms thought so that neither is ever tested. The two loops interlock — avoidance prevents disconfirmation, and undisconfirmed cognition sustains avoidance — which is why cognitive and behavioural methods are one therapy and not two.

Key concepts

HOT THOUGHT

The cognition that carries the affect: not the catalogue of what the child thinks but the one that, when spoken aloud, moves the feeling. In the room: the sentence you ask the child to say out loud and then rate — "and if that were true, what would it mean about you?"

COGNITIVE DISTORTION

A systematic error in inference — catastrophising, mind-reading, all-or-nothing thinking, overgeneralisation, personalisation, emotional reasoning. In the room: a hypothesis about the thinking, not a verdict on the thinker; children detect being corrected and close.

BEHAVIOURAL EXPERIMENT

A test the child designs and runs: prediction stated, action taken, outcome compared with prediction. The engine of cognitive change, because it lets reality do the arguing. In the room: the difference between "let's think about whether that thought is true" and "let's find out".

SAFETY BEHAVIOUR
What the child does to prevent the feared catastrophe — the whispered rehearsal, the gripped pen, the scanned exit. In the room: the reason exposure sometimes fails: the catastrophe is never tested because the safety behaviour is credited with the non-occurrence.

Lecture I — The model, and why feeling is not evidence

Proposition 1. Between situation and response there is an appraisal, and the appraisal carries the affect. Two children called to the board feel differently because they think differently: "they will see me freeze" and "I get to show the sum I know" are different events. The model is not that thoughts cause everything; it is that appraisals are a legitimate, changeable part of the chain. Figure 6.1 draws the chain and the shortcut by which feeling is recruited as evidence for the thought.

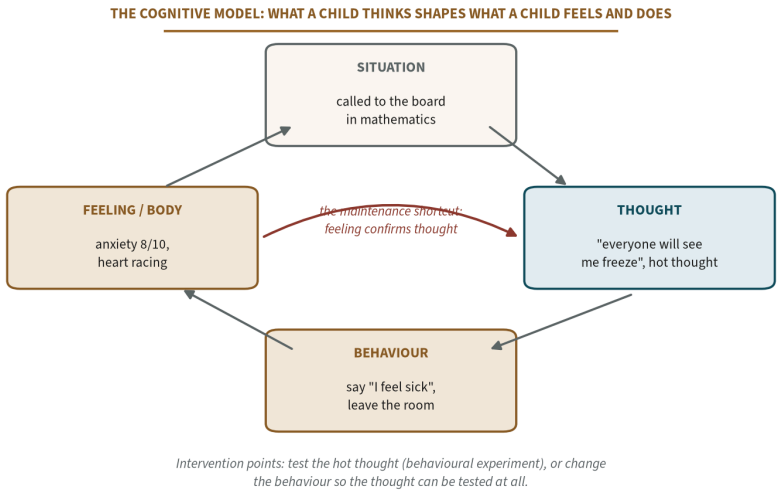


Figure 6.1 The cognitive model as a cycle: situation, thought, feeling and body, behaviour, and back again, with the maintenance shortcut by which the feeling is taken as confirmation of the thought. Intervention points are the test of the hot thought and the change of behaviour that makes testing possible. *Source:* prepared for this edition.

Proposition 2. Avoidance protects the cognition. A thought that is never tested cannot be disconfirmed, and every act of avoidance is a small experiment whose result is "the danger was real, and I escaped it". This is the cognitive reason the behavioural loop of week 5 is so stable, and the reason exposure must drop safety behaviours to work.

Proposition 3. Development gates the method. The cognitive model requires the child to hold a thought as an object of inspection — a metacognitive capacity that develops through middle childhood and is unreliable before about

seven, and that adolescence complicates with self-consciousness. With a six-year-old you do not dispute thoughts; you change what happens and let new experience do the teaching, or you externalise the thought as a character ("the worry bully") so it can be talked to. With a ten-year-old you can catch, check and change with concrete materials. With a fifteen-year-old you can work with rules and assumptions ("if I am not first, I am nothing"), which is where the durable change usually lies.

Proposition 4. The traps are teachable. Figure 6.2 collects the six most common. Taught as a game of spotting — in stories, in cartoons, in the counsellor's own examples first — they give the child a vocabulary for their own inference errors without shame.

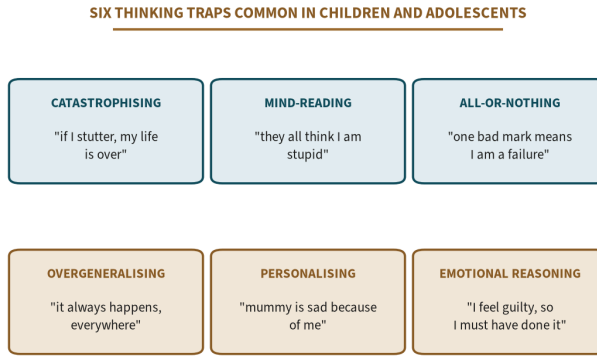


Figure 6.2 Six thinking traps common in children and adolescents — catastrophising, mind-reading, all-or-nothing, overgeneralising, personalising and emotional reasoning — each illustrated in a child's idiom. Each trap is a hypothesis to test rather than a fault to correct. *Source:* prepared for this edition.

> **PRACTICE NOTE**
> **Clinical implication.** Never dispute a child's thought directly. Disputation teaches compliance: the child learns to produce the "rational" sentence while keeping the belief. Test it instead, and let the result belong to the child.

Worked example 6.1 — From verbatim quote to experiment

Task. Amaka (10), before a class presentation: "If my voice shakes, everyone will laugh, and then I'll never be able to come back to class." Identify the hot thought, the traps, and design a behavioural experiment she could run this week.

Solution. Hot thought: "everyone will laugh" (the voice shaking she accepts; the universal laughter is the catastrophe). Traps: mind-reading (she knows what

everyone will think), catastrophising (laughter becomes permanent exclusion), and all-or-nothing in the "never be able to come back". Experiment, co-designed: prediction written in her words and rated 0–100 ("80% will laugh out loud"); test — she asks two classmates afterwards what they noticed, and the counsellor (with consent) observes from the back and counts actual audible laughs; comparison — predicted number versus counted number, and what the classmates said. Disconfirmation criterion agreed in advance: fewer than three audible laughs, or classmates reporting they noticed nothing. Safety behaviours to drop: gripping the lectern, reading without looking up. The follow-up question is the durable one: "if nobody laughed, what does that tell the thought?" — because a single disconfirmation without a revised belief is a lucky day, not a change.

Lecture II — Methods, in the order a child can use them

Catch it. Externalise and slow: the thought is written or drawn, given a voice, a size, a colour. With younger children a "thought bubble" prop is enough; the point is distance.

Check it. Not "is it true?" but "how could we find out?", "what's the evidence for and against, in things that actually happened?", "what would you tell a friend who had this thought?" The friend question reliably produces better reasoning than the self question, and the difference is itself data about the child's self-relationship.

Change it — or change the behaviour. Sometimes the cognition yields to a better one ("some people might look; looking isn't laughing; and I can finish the sentence even if they look"). Sometimes the useful move is behavioural: do the avoided thing with the thought still present and see what happens. With children, the second route is usually the stronger, because experience is more persuasive than argument and needs less metacognition.

Consolidate. A coping card in the child's own words, one sentence, carried into the next test. Not a slogan supplied by the adult: a sentence the child wrote after an experiment that went their way.

> **SAFEGUARDING AND DUTY TO REPORT**
> Cognitive work can surface content that is not a distortion but a fact: a child who thinks "my stepfather hits me and nobody stops it" is not catastrophising. The discipline of the week is to test thoughts, and the discipline of the profession is to recognise when the thought is an accurate report. Where a cognition describes harm, the safeguarding procedure governs and the cognitive work pauses. Do not let a method make you deaf.

Lecture III — What the evidence supports

Cognitive-behavioural therapy is the best-evidenced psychological treatment for childhood anxiety disorders: manualised programmes built on exposure plus cognitive work outperform waitlist and attention controls, with effects that hold at follow-up, and computer-assisted and parent-involved variants extend reach. Kendall's programme is the standard exemplar. For adolescent depression the picture is weaker: CBT outperforms waitlist and sometimes treatment-as-usual, effects are modest, and for moderate depression the combination with careful medical and social assessment is the norm rather than therapy alone. For conduct problems, cognition alone is not the treatment — the evidence sits with the parents and the contingencies (week 5). Three cautions complete the picture: effects are smaller in routine clinics than in trials; dropout and non-response are common, and measuring outcome session by session (week 14) is how you notice; and the "cognitive" ingredient's specific contribution is smaller than the exposure and the relationship, which is an argument for humility about the ingredient and confidence about the package.

> **PRACTICE NOTE**
> **Clinical implication.** Match the method to the disorder and the age: exposure-led for anxiety, contingency-led for conduct, combined and modest for depression, and cognitive language only where the child's development can carry it. A thought record given to a seven-year-old is not treatment; it is paperwork.

Worked example 6.2 — Adapting down and up

Task. The same hot thought — "if I get it wrong, I'm stupid" — appears in Musa (6), Kemi (10) and Tobi (15). Specify the method for each.

Solution. Musa (6): no disputation, no record. Change the contingency and the model: errors are celebrated loudly in the room ("mistakes mean your brain is growing"), the adult models their own errors and repair, and success is shaped in small steps so that "wrong" is frequent and safe. The cognition changes by experience, over months. Kemi (10): catch-check-change with materials — the thought on a card, evidence for and against in two columns of real events, one behavioural experiment a week (answer a question she might get wrong, on purpose, and count the consequences), coping card in her words. Tobi (15): work at the rule level: the assumption "worth = performance"; downward arrow to the meaning ("if I'm stupid I'm unlovable"); behavioural experiments against the rule (disclose a failure and observe the relationship); and a values conversation about what he wants a mind for. Same thought, three developments, three methods.

SKILLS LABORATORY

Practical 6 — the experiment clinic. In pairs, one plays a child supplying a real-sounding hot thought (cards supplied), the other has ten minutes to produce, with the "child", a written experiment: prediction and rating, test, disconfirmation criterion, safety behaviours to drop, and the follow-up question. The observer checks: was the prediction the child's words? was disconfirmation defined before the test? was any safety behaviour left in? Rotate. Debrief on which checks you habitually miss.

ASSESSMENT

Assessed task. A cognitive formulation and experiment design of 1,200 words on a supplied case: the model drawn with the child's own material, hot thought and traps identified from verbatim quotes, one experiment fully specified, and a developmental justification of the methods chosen.

Criterion	Weight
Hot thought is the child's words and carries the affect	20%
Experiment specified (prediction, test, disconfirmation, safety behaviours)	30%
Developmental justification of method	25%
Model drawn correctly with the maintenance shortcut	15%
APA 7th and prose quality	10%

Formative. Bring one verbatim hot thought you have heard from a child, with the trap you judge it to instantiate.

UNIT SYNTHESIS

Synthesis. Cognition is the second loop: appraisal carries affect, avoidance protects appraisal, and the behavioural experiment — reality, borrowed for an afternoon — is the instrument that opens the loop, provided the method matches the child's development and the disorder.

READING (APA 7TH)

- Beck, A. T. (1976). *Cognitive therapy and the emotional disorders*. International Universities Press.
- Friedberg, R. D., & McClure, J. M. (2015). *Clinical practice of cognitive therapy with children and adolescents: The nuts and bolts* (2nd ed.). Guilford Press.
- Kendall, P. C., & Hedtke, K. A. (2006). *Cognitive-behavioral therapy for anxious children: Therapist manual* (3rd ed.). Workbook Publishing.

- Stallard, P. (2002). *Think good — feel good: A cognitive behaviour therapy workbook for children and young people*. John Wiley & Sons.
- Weisz, J. R., & Kazdin, A. E. (Eds.). (2017). *Evidence-based psychotherapies for children and adolescents* (3rd ed.). Guilford Press. — **core text**; the anxiety and depression chapters.
- Creswell, C., & Cartwright-Hatton, S. (2007). Family treatment of child anxiety: Outcomes, limitations and future directions. *Clinical Child and Family Psychology Review, 10*(3), 232–252.

Seminar questions

1. Why does direct disputation produce compliance rather than change? Give the mechanism and one clinical sign that it has happened.
2. A child's hot thought is true (they are, in fact, behind in reading). What is the cognitive work, and what is not?
3. Where does the feeling-as-evidence shortcut appear in your own reasoning about cases? Give an example.
4. Is a coping card a cognitive intervention or a self-instruction? Does the distinction matter for how you build it?
5. If the specific cognitive ingredient contributes little beyond exposure and alliance, should programmes still teach cognitive method? Argue both sides.

Next week

Week 7 completes the behavioural lineage with the third wave: acceptance and commitment therapy, dialectical behaviour therapy for adolescents, and mindfulness-based work with young people. Read the six processes of psychological flexibility before the session and bring one example of a child trying to control an uncontrollable inner event.

7 LECTURE UNIT 7 Teaching week 7 4 notional hours

The Behavioural Lineage III: The Third Wave — Acceptance, Dialectics, and Mindfulness with Young People

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Explain** what the third wave retains from CBT and what it rejects, naming the control agenda as the shared target.
2. **Describe** the six processes of psychological flexibility and locate one in a session excerpt.

3. **Demonstrate** two defusion exercises suitable for a child of eight and one suitable for a fifteen-year-old.
4. **Outline** the structure and rationale of DBT skills training for adolescents, including the function of phone coaching and of the therapist team.
5. **State** the current state of the evidence for ACT, DBT-A and mindfulness-based interventions with young people, including the replication concerns.

Where this week sits

Weeks 5 and 6 built a therapy of change: analyse the loop, test the thought, alter the consequence. This week asks what happens when the loop cannot be changed, or when the attempt to change inner experience is itself the loop. The third wave does not replace CBT; it notices that a child fighting their own mind is a child in a fight, and offers a different relationship to inner events as the mechanism.

Key concepts

THE CONTROL AGENDA
The strategy of treating unwanted thoughts and feelings as things to be eliminated, and measuring success by their absence. In the room: the adolescent who has spent four years trying not to feel panic and has organised a life around the trying. The third wave's first move is to show that the strategy, not the feeling, is the problem.

DEFUSION
Changing the relationship to a thought without changing its content: from "I am stupid" to "I am having the thought that I am stupid", and then to "my mind is offering the stupid story again". In the room: the difference between arguing with a thought and watching it arrive.

PSYCHOLOGICAL FLEXIBILITY
The capacity to stay in contact with the present, with what one feels, while doing what one values. Not symptom removal; the outcome variable of ACT and, Figure 7.1 argues, of much good therapy under another name.

RADICAL ACCEPTANCE
In DBT, the deliberate stopping of the fight with what is already true — not approval, not resignation, but the end of the extra suffering that fighting adds. In the room: what makes room for change, paradoxically, because a child who is no longer at war with their history can plan a future.

Lecture I — What the third wave noticed

Proposition 1. Thought suppression rebounds. Instruct a person not to think of something and the thought returns with interest; for anxious children, the attempt to control inner events increases their salience and the fear of them. A therapy whose method is control of inner events can therefore manufacture the problem it treats.

Proposition 2. The content of the thought may matter less than the stance toward it. Two children hold "I am worthless". One is fused with it — it is simply true, the way the floor is there. The other can watch it: "that's the old tape". Same content, different suffering, different prognosis. Defusion trains the second stance without disputing the content, which is why it works where disputation produced compliance (week 6).

Proposition 3. Values give the work a direction that symptoms cannot. A life organised around not feeling anxious shrinks; a life organised around being a friend, a player, a maker expands and carries discomfort with it. Committed action toward a valued direction is the behavioural engine of ACT, and it is measurable in ways feelings are not.

Proposition 4. Dialectics adds the other pole: change. DBT's structure is a held tension — acceptance of the child exactly as they are, and relentless teaching of skills to build a life worth living. For adolescents with self-harm and emotional dysregulation, the package includes skills groups (mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness), individual therapy, phone coaching at the moment of crisis, and — crucially — a consultation team for the therapists, because the work punishes isolated practitioners.

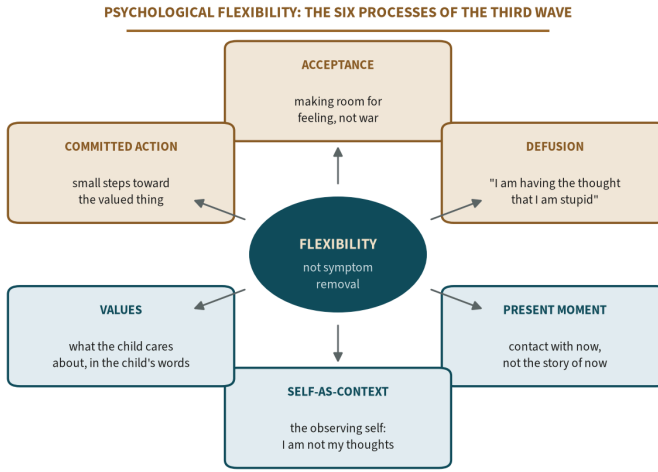


Figure 7.1 The six processes of psychological flexibility arranged around flexibility itself: acceptance, defusion, present-moment contact, self-as-context, values and committed action. After Hayes, Strosahl and Wilson (1999); with adolescents the processes are taught in the adolescent's own language rather than the diagram's. *Source:* prepared for this edition.

PRACTICE NOTE

Clinical implication. Ask of any young client: what has this child been trying not to feel, and what has the trying cost? The answer is usually the case formulation, and it is often not the presenting complaint.

Worked example 7.1 — The cost of control

Task. Ngozi (15) presents with "anger problems": outbursts at home, one suspension. History: father left when she was nine; mother works double shifts; Ngozi cares for two younger siblings. She describes the outbursts as "the volcano", and her strategy as counting to ten, squeezing a stress ball, and leaving the room — strategies that "never work". Using the week's frame, formulate.

Solution. The control agenda is the organisation of her life: the volcano must not erupt, every tool is aimed at suppression, and every failure of suppression is evidence that she is defective. What she has not been helped to feel is grief and loneliness at nine, and exhaustion and unfairness now. Defusion target: "I am a volcano" (a fused identity, not a thought about a feeling). Values direction, in her words: being the sister her siblings run to, and being someone who finishes things. Committed action candidates: a weekly hour that is hers; telling one person at school one true thing. Acceptance work: the outburst is not first a discipline problem but the only available expression of an unheld feeling; the skills (distress tolerance, opposite action) are taught *after* the feeling has been met,

or they become new tools of suppression and will "never work" like the others. Note what the formulation refuses: a diagnosis of conduct problem, and a plan of consequences.

Lecture II — Methods with children, and the discipline of language

The third wave's vocabulary is adult and abstract; with children it must be translated or it fails. Four translations carry most of the work.

Defusion as play. Give the thought a voice (the counsellor's silly voice, a puppet's), sing it, write it on a leaf and watch it float, thank the mind for the suggestion. The mechanism is identical to the adult exercise — the thought becomes an object — and the medium is the child's.

Acceptance as making room. The struggle switch: a two-position switch the child can imagine, "fighting it" and "letting it be here while I get on with my day". With a nine-year-old, a physical metaphor works: holding a beach ball under water takes both arms and all attention; letting it float beside you costs nothing and it is still there.

Values as a compass, not goals. Goals can be achieved and ticked; values are directions. With adolescents, the values conversation is often the first time anyone has asked what they are for, and it is where engagement is won.

Mindfulness as attention training, stripped of claims. Noticing one thing fully (a raisin, a sound, the breath at the nose), noticing the mind wander, returning without comment. The return is the repetition that trains; the wandering is not failure. Secular, brief, and practiceable in a classroom — which is why school-based programmes adopted it, and why the evidence question matters.

> **SAFEGUARDING AND DUTY TO REPORT**
> Mindfulness and acceptance exercises can increase arousal and intrusive material in traumatised children: eyes-closed body scans in particular. With any child with a trauma history, use eyes-open, externally anchored practice (sound, object, movement), keep exposures short, and stop at the first sign of flooding. The method is not neutral and its dosage is clinical judgement.

Lecture III — The evidence, honestly stated

For adolescents with recurrent self-harm and emotional dysregulation, DBT-A has the strongest third-wave evidence: trials show reductions in self-harm frequency and improvements in emotion regulation against comparison conditions, and the package's structure (skills group, individual work, coaching, therapist team) is itself a teaching model for services. For ACT with young people, randomised trials exist across anxiety, depression, pain and chronic illness, with effects generally comparable to CBT and superior to waitlist; the mechanism evidence — that change runs through flexibility — is promising but thinner

than the outcome evidence. For mindfulness-based interventions with children and adolescents, early enthusiasm outran the data: school-based universal programmes show small and inconsistent effects on wellbeing and negligible effects on symptoms in the largest well-conducted trials, and clinician-delivered mindfulness-based therapy for selected young people does better. Three conclusions for practice: use DBT-A as a package where it is indicated, not as a skills sheet; use ACT as a full therapy with a formulation, not as metaphors borrowed for other work; and treat universal school mindfulness as a promising public-health experiment rather than a treatment.

PRACTICE NOTE

Clinical implication. Borrowing third-wave techniques into another therapy is common and often useful; claiming third-wave outcomes from borrowed techniques is not. Know which you are doing, and write it in the plan.

Worked example 7.2 — One excerpt, two stances

Task. In session, Ibrahim (13) says: "I tried the breathing thing and I still felt like a failure, so it doesn't work." Classify the stance and write your response in ACT-consistent terms.

Solution. The stance is the control agenda applied to the method: the breathing was recruited to eliminate "feeling like a failure", and its failure to eliminate is read as the method's failure. An ACT-consistent response notices the work the thought is doing and returns the method to its actual function: "so the mind offered 'it doesn't work' the moment the feeling stayed. What if the breathing isn't for making the feeling go? What if it's for staying in the room with it, so you can do the next thing?" Then, behaviourally: what was the next thing he wanted to do that afternoon, and did he do it? The outcome variable moves from feeling to action, which is the week's central reorientation.

SKILLS LABORATORY

Practical 7 — defusion laboratory. In triads. Each member supplies (from a card or their own material) one fused self-statement. The group has five minutes per statement to devise and run a defusion exercise appropriate to (a) an eight-year-old and (b) a fifteen-year-old. Criteria: the content is unchanged; the relationship to it changes; nothing is disputed; the child's dignity is intact. Debrief: which exercises felt like tricks, and why do tricks fail?

ASSESSMENT

Assessed task. A third-wave formulation of 1,200 words on a supplied adolescent case: the control agenda and its costs, one fused cognition and a defusion plan

matched to age, a values direction in the young person's idiom, two committed actions, and a statement of what in the case the third wave does not address.

Criterion	Weight
Control agenda identified with its costs	25%
Defusion plan age-matched and content-preserving	25%
Values direction in the young person's own idiom	20%
Committed actions concrete and valued-directed	15%
Limits of the approach stated	15%

Formative. Submit one example of a child trying to control an uncontrollable inner event, with what the control has cost.

UNIT SYNTHESIS

Synthesis. The third wave's contribution is a change of target: from the presence of difficult inner events to the child's relationship with them and their movement toward what they value — and its discipline is knowing when that is a therapy and when it is a borrowed metaphor.

READING (APA 7TH)

- Coyne, L. W., McHugh, L., & Martinez, E. R. (2011). Acceptance and commitment therapy (ACT): Advances and applications with children, adolescents, and families. *Child and Adolescent Psychiatric Clinics of North America, 20*(2), 379–399.
- Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (1999). *Acceptance and commitment therapy: An experiential approach to behavior change*. Guilford Press.
- Linehan, M. M. (1993). *Cognitive-behavioral treatment of borderline personality disorder*. Guilford Press.
- Rathus, J. H., & Miller, A. L. (2015). *DBT skills manual for adolescents*. Guilford Press. — **core text**; the skills modules and the adaptations for adolescents.
- Dunning, D., Griffiths, K., Kuyken, W., Jones, E., Foulkes, L., Parker, P., & Dalgleish, T. (2019). Research review: The effects of mindfulness-based interventions on cognition and mental health in children and adolescents — A meta-analysis of randomized controlled trials. *Journal of Child Psychology and Psychiatry, 60*(3), 244–258.
- Swales, M. A. (Ed.). (2019). *The Oxford handbook of dialectical behaviour therapy*. Oxford University Press.

Seminar questions

1. Is defusion a cognitive technique under another name? What turns on the answer?

2. A parent hears "acceptance" as "giving up". Compose the correction in ordinary language.
3. Why does DBT insist on a therapist consultation team? What does the absence of one predict?
4. Universal school mindfulness shows small effects. Should schools stop? Argue from the evidence and from the function of schools.
5. Where is the line between radical acceptance and colluding with an unacceptable situation — in a family, in a school, in a care system?

Next week

Week 8 changes lineage again: the person-centred tradition, the three conditions, and the claim that relationship is not the context of therapy but the therapy. Read Rogers's statement of the conditions and bring one session in which you believe the relationship itself did the work.

8 LECTURE UNIT 8 Teaching week 8 4 notional hours

The Humanistic Lineage I: Person-Centred — The Conditions and the Child

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **State** Rogers's three conditions and his sufficiency claim, and distinguish the claim from the weaker claim that the conditions help.
2. **Demonstrate** reflective responding at three levels — content, feeling, and meaning — in a timed micro-practice.
3. **Explain** what child-centred play therapy adds to, and removes from, adult person-centred practice.
4. **Evaluate** the evidence for the conditions and for non-directive play therapy, including what the common-factors literature of week 1 does and does not settle.
5. **Identify** two situations in which a non-directive stance is contraindicated or insufficient with a child, and say what you would do instead.

Where this week sits

Three lineages down, one question has been constant: what does the practitioner actually do? The behavioural lineage answers with contingencies, the cognitive with experiments. This week's lineage answers with a way of being, and makes the strongest claim in the course: that under specifiable relational conditions, the child does the therapeutic work themselves, and the adult's techniques are at best

unnecessary and at worst an interference. You are entitled to reject the claim. You are not entitled to skip it, because the conditions it names are the ones every other lineage borrows when its techniques stall.

Key concepts

CONGRUENCE
The adult is internally what they externally appear; no professional mask, no performed calm. Not self-disclosure of everything — the adult's material is used only where it serves the child — but the absence of a facade the child can sense and must manage. In the room: the child who relaxes when you admit, plainly and briefly, that you too found that hard.

UNCONDITIONAL POSITIVE REGARD
Prizing the child without conditions: not approving every behaviour, not pretending the behaviour has no consequences, but holding the person as of worth while the behaviour is addressed. In the room: the difference between "what you did hurt him and we will deal with that" delivered inside an unbroken relationship, and the same words delivered as withdrawal.

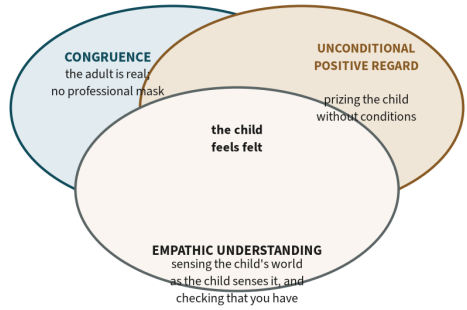
EMPATHIC UNDERSTANDING
Sensing the child's private world as the child senses it, and checking that you have — the checking being half of it. Not sympathy (feeling for), not mind-reading (feeling instead of). In the room: the reflection that ends in a question mark the child can correct, and the correction being welcomed as data.

NON-DIRECTIVENESS
The deliberate refusal to steer: no advice, no interpretation, no curriculum of questions; the child chooses the material and the pace. In the room: the hardest discipline in the course, because helpfulness is the practitioner's favourite vice.

Lecture I — The claim, precisely stated

Proposition 1. Rogers's claim is conditional and strong. If the adult is congruent, holds unconditional positive regard, and understands the child's inner world empathically — and the child perceives at least a minimum of these — then constructive personality change occurs. Not "the conditions help the techniques work"; the conditions, perceived, are sufficient. Figure 8.1 draws the claim and the question mark this course attaches to it.

THE THREE CONDITIONS: NECESSARY AND SUFFICIENT, OR ASPIRATIONAL?



Rogers claimed the conditions are sufficient for change. With children the claim needs the parent in the room: conditions for whom?

Figure 8.1 The three person-centred conditions as overlapping circles — congruence, unconditional positive regard and empathic understanding — meeting in the child's experience of being felt, with the open question of whether the conditions are sufficient for change or its necessary context. *Source:* prepared for this edition.

Proposition 2. The mechanism is the reversal of conditions of worth. The child who learned that regard is conditional — lovable when quiet, when achieving, when not angry — organises the self around the conditions and disowns what they exclude. Therapy supplies the opposite experience: regard that does not move. Under it, the disowned material can re-enter without catastrophe, and the organism's actualising tendency resumes its work. The mechanism is testable in a soft sense: predicted changes in self-description and in the handling of previously denied feeling.

Proposition 3. With children, the medium changes and the stance survives. A child cannot sustain reflective dialogue. Child-centred play therapy (Axline's eight principles, formalised by Landreth) keeps the conditions and moves them into play: the room is permissive within firm limits, the child leads, the adult tracks and reflects play and feeling ("you're making him very small"), and does not teach, direct, or hurry. The limits are themselves part of the regard: a room with no limits is not safe, and a child knows it.

Proposition 4. Reflection is a skill with levels, not a parrot trick. Reflecting content ("the tower fell") is tracking; reflecting feeling ("you're furious it fell") is empathy at the first level; reflecting meaning ("it always falls just when it's nearly finished") is empathy at the level the child cannot yet reach alone, and is the one that moves therapy. Mis-timed deep reflection is experienced as intrusion; shallow reflection as boredom. Calibration is the craft.

PRACTICE NOTE

Clinical implication. Test your reflections by the child's response, not by your intention. A reflection that lands produces more material or a correction; a reflection that misses produces politeness or silence. Politeness from a child is a missed reflection until proved otherwise.

Worked example 8.1 — Three levels of reflection

Task. Halima (7) is playing with a doll family. She makes the mother doll leave the house carrying a bag, then has the child doll stand at the window for a long time doing nothing. She says nothing. Write one reflection at each level, and state the risk of each.

Solution. Content: "She's going away with her bag, and he's watching from the window." Risk: none, but nothing moves; used too long it becomes narration the child must tolerate. Feeling: "He's waiting. It's a long wait." Risk: moderate — you have named an affect the child has not shown; if wrong, she will correct you, and the correction is progress. Meaning: "He waits at the window every time, in case she comes back that way." Risk: high — this is an interpretation wearing reflection's clothes; if the play is not about return, you have put your story in her room. Sequence matters: earn the deep reflection with two that landed. Note what non-directiveness forbids here: asking who the dolls are, asking whether this is like her home, and reassuring her that mummies come back. All three would convert her play into your agenda.

Lecture II — What the evidence says, and what it cannot say

The conditions are among the best-supported findings in psychotherapy research, though not in the form Rogers stated. Relationship variables — alliance, empathy, goal consensus, positive regard — predict outcome across therapies and across client ages, and the adult alliance in child work predicts outcome modestly while the child's own alliance predicts it better; Norcross and Lambert's summaries are the standard citation. That supports the weaker claim: the conditions matter enormously. It does not settle the sufficiency claim, because no design can isolate perceived conditions from everything else a practitioner does.

For non-directive child-centred play therapy the evidence is mixed: against waitlist it shows effects on internalising difficulties and on self-concept; against active treatments it generally does not show superiority, and several positive studies are small and allegiance-laden. Two honest conclusions follow. First, non-directive play therapy is a legitimate treatment for some children — particularly those whose difficulties are organised around regard, control and expression, and those too young for verbal method. Second, it is not a default, and presenting it as the gentle option for every young child mis-states the evidence.

SAFEGUARDING AND DUTY TO REPORT

Non-directiveness is a therapeutic stance, not a safeguarding stance. Where a child's play or speech discloses risk, the directive duties of the profession override the stance completely: you stop following the child's lead and you act on procedure. A child-centred room is permissive within limits, and the limits include the law. Practitioners who confuse the stance with passivity in the face of risk cause the harm this lineage exists to prevent.

Lecture III — Where non-directiveness is not enough

Three situations require you to say so plainly and change method.

Where the difficulty is maintained by contingencies the child cannot change. Enuresis maintained by night-time waking patterns (week 3), school refusal maintained by morning contingencies (weeks 1 and 5): reflective understanding of the child's experience is necessary and will not move the loop. The plan needs the adults and the contingencies.

Where the child needs skills they do not have. Emotion-regulation deficits, social-skill deficits, trauma-processing deficits: acceptance and prizing do not teach distress tolerance or narrative reconstruction. The humanistic stance makes the teaching possible; it is not the teaching.

Where safety requires direction. Risk of harm to self or others, disclosure of abuse, acute crisis: the session becomes directive, structured and, where required, reported. The conditions remain — congruence, regard, empathy — inside a frame the child is told about honestly.

The mature position is not eclecticism but sequence and honesty: know what each element of your work is for, which lineage supplies it, and what would show it failing. That is the position week 13 builds.

PRACTICE NOTE

Clinical implication. If you find yourself unwilling to give advice because the stance forbids it, while the child drowns in a contingency they cannot change, you are serving the theory rather than the child. Name the element you are using and why; the child's interest governs, not the lineage's pride.

Worked example 8.2 — The parent's question

Task. A father asks, after six non-directive sessions with his son (6): "You just sit there and describe what he does. When does the therapy start?" Compose your reply in ordinary language, under ninety seconds, naming the mechanism without jargon.

Solution. "What your son is doing in there is the therapy, and my job is harder than it looks. For six years he has learned which parts of himself get approval and

which get him into trouble, so the troublesome parts went underground — into the wetting, the rages, the refusal. In that room nothing he feels gets him into trouble with me, and nothing he plays is corrected. That is not nothing: it is the first relationship in which the whole of him is allowed to be present, and children change from the inside outwards when that happens. What you will see first is not insight — he is six — it is the play getting louder and messier before it gets calmer, and the rages at home easing as they stop being necessary. If by week ten we see neither, I will tell you, and we will change the plan together." Note the structure: mechanism (regard without conditions re-admits disowned material), prediction (play intensifies then settles; home behaviour eases), and a falsification date. A parent can trust a plan that can be wrong.

SKILLS LABORATORY

Practical 8 — reflective micro-practice. In pairs, six minutes each way. One speaks about a mildly difficult real event of their own (not a client); the other reflects only — content, feeling, meaning — with no questions, no advice, no reassurance, no self-disclosure. The speaker marks afterwards: which reflections landed, which missed, which felt like intrusion, and whether the final minute went deeper than the first. Swap. Debrief: what did you want to do that you were forbidden from doing, and what happened when you didn't do it?

ASSESSMENT

Assessed task. A recorded or transcribed eight-minute micro-practice with a peer, submitted with a 600-word commentary: identify three reflections by level, one miss and what the speaker's response showed, one moment where directive pull arose and how it was handled, and a statement of one case type for which you would not use a non-directive method.

Criterion	Weight
Reflections at three levels present and calibrated	35%
No questions, advice or reassurance in the recorded segment	20%
Commentary uses the speaker's feedback as data	20%
Contraindication stated with reasons	15%
Prose quality	10%

Formative. Bring one session in which you believe the relationship itself did the work, with the observation that supports the belief.

UNIT SYNTHESIS

Synthesis. The person-centred lineage's claim is that perceived congruence, unconditional positive regard and empathic understanding are sufficient for change; the evidence secures the weaker and more useful truth that they are the ground on which every other method stands — and non-directiveness is a discipline to be chosen, not a default to hide in.

READING (APA 7TH)

- Axline, V. M. (1947). *Play therapy: The inner dynamics of childhood*. Houghton Mifflin.
- Landreth, G. L. (2012). *Play therapy: The art of the relationship* (3rd ed.). Routledge. — **core text**; the child-centred method and the eight principles.
- Norcross, J. C., & Lambert, M. J. (2019). Psychotherapy relationships that work III. *Psychotherapy, 56*(4), 503–515.
- Rogers, C. R. (1951). *Client-centered therapy: Its current practice, implications, and theory*. Houghton Mifflin.
- Rogers, C. R. (1961). *On becoming a person: A therapist's view of psychotherapy*. Houghton Mifflin.
- Wilson, K., & Ryan, V. (2017). *Play therapy with children and adolescents in crisis: Individual and group treatment manual* (4th ed.). Brunner-Routledge (consult the current edition for crisis adaptations).

Seminar questions

1. Rogers claimed sufficiency; the evidence secures importance. What, practically, is lost and gained by holding the weaker claim?
2. Is congruence compatible with professional boundaries? Give a case where disclosure serves the child and one where it serves the adult.
3. A child-centred playroom has firm limits. Reconcile limits with unconditional positive regard in one paragraph a parent could read.
4. Why is politeness from a child evidence of a missed reflection? What else could it be evidence of?
5. Which of this week's three contraindications would be hardest for you personally to act on, and what would you need in order to act?

Next week

Week 9 completes the humanistic lineage with existential and meaning-focused work with adolescents: the givens of existence, meaning in adversity, and what a counsellor does with a fifteen-year-old who asks whether anything matters. Read Frankl's account of meaning in extremity and bring one question of this kind that a young person has asked you.

9 LECTURE UNIT 9 Teaching week 9 4 notional hours

The Humanistic Lineage II: Existential Counselling and Meaning Work with Adolescents

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Name** the four existential givens and give, for each, the form it typically takes in adolescence.
2. **Distinguish** a philosophical meaning question, a depressive cognition, and a statement of suicidal intent, from three near-identical utterances, and state the different obligation each creates.
3. **Conduct** a meaning-in-adversity conversation using at least three of Frankl's routes (creation, experience, attitude) without resorting to reassurance.
4. **Explain** what existential counselling adds to, and refuses from, technique-based practice.
5. **State** the limits of existential method with children below adolescence, and what you would use instead.

Where this week sits

Week 8 located change in the relationship. This week locates the material that relationship must eventually hold: the questions that have no technical answer. Adolescents ask them plainly — *what is the point, why does anyone bother, does it matter that I die?* — and the professional reflex is to treat the question as a symptom. Sometimes it is. Often it is the healthiest thing in the room, and the adolescent's first test of whether an adult can bear reality. This week teaches you to bear it, and to know precisely when the question stops being philosophical and becomes a risk assessment.

Key concepts

THE GIVENS

The conditions no therapy removes: mortality, freedom (and its responsibility), existential isolation, and the absence of guaranteed meaning. In the room: not topics to be resolved but ground to be acknowledged. An adolescent who meets them with company develops; alone with them, they are at risk.

WILL TO MEANING

Frankl's claim that the primary human motivation is not pleasure or power but meaning, and that meaning remains available in suffering through three routes: what

we create, what we experience and love, and the attitude we take toward what cannot be changed. In the room: the three doors you can open with a young person whose circumstances will not improve this term.

TRAGIC OPTIMISM
Optimism that has looked at the tragedy and stayed: saying yes to life in spite of pain, guilt and death. Not positivity, which is its counterfeit and is detected instantly by adolescents. In the room: the difference between "it will get better" and "this is awful, and you are still here, and what you do with it is still yours".

EXISTENTIAL ANXIETY
Anxiety in the face of a given, as distinct from neurotic anxiety disproportionate to its object. In the room: the dread before an exam that is about the exam, and the dread before an exam that is about being a self who can fail in front of others for the rest of a finite life. Only the second is existential, and only the first responds to study skills.

Lecture I — The givens, in adolescent form

Proposition 1. Adolescence is when the givens arrive. The child lives in a world whose meanings are supplied; the adolescent discovers that the supply was local and contingent. Death becomes personal ("I will not be here, and the world will continue without me, which means I was optional"); freedom becomes visible ("no one is actually choosing for me, and the choosing does not stop"); isolation becomes literal ("no one can be inside my head, and the people who claim to understand me are guessing"); and meaning becomes a question rather than a background.

Proposition 2. The questions are developmental, not diagnostic. A fifteen-year-old who asks whether anything matters is, in the ordinary case, doing the work of the age. Pathologising the question teaches the adolescent that reality-talk is symptomatic, and they will stop bringing reality. The clinical task is first to establish which case you are in — philosophical, depressive, or at-risk — and Figure 9.1 sets the four givens in the idiom in which they actually arrive.

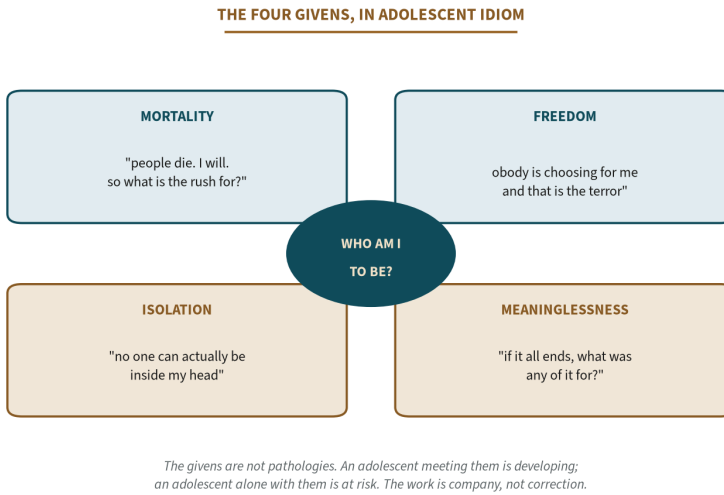


Figure 9.1 The four existential givens in adolescent idiom — mortality, freedom, isolation and meaninglessness — arranged around the central adolescent question of who one is to be. The givens are not pathologies; an adolescent meeting them is developing, an adolescent alone with them is at risk. *Source:* prepared for this edition.

Proposition 3. Identity is the givens' meeting place. The question "who am I?" is freedom experienced from the inside: the self as a task rather than a fact. Existential counselling reads identity work not as a stage to be completed but as a stance to be owned — and reads identity foreclosure (the self adopted wholesale from family, faith or gang without examination) as a defence against freedom, to be respected and, in time, questioned.

Proposition 4. Existential method is a stance, not a toolkit. There is no technique manual because the position is that no technique reaches the givens; what reaches them is a relationship in which reality can be said aloud. The discipline is therefore negative as much as positive: no reassurance that cancels the given, no interpretation that reduces it, no hurry.

> **PRACTICE NOTE**
> **Clinical implication.** When an adolescent states a given, your first response should acknowledge it as true before anything else happens. "Yes — nobody can be inside your head. That is real, and it is lonely, and you bringing it here is not a symptom." Reassurance first is a refusal of company.

Worked example 9.1 — Three utterances, three obligations

Task. Classify each and state your next action. (a) Tunde (16): "Honestly, does any of it matter? We'll all be dead in a hundred years and nobody will remember the essay." (b) Amara (15): "Nothing matters. Not in a deep way. In the way that

getting up is pointless and I'd rather not." (c) Kele (17): "Nothing matters. Which is fine, because it won't be a problem for much longer."

Solution. (a) Philosophical: the affect is lively, the framing general, the invitation argumentative. Obligation: take the argument seriously — sit with it, offer company and counter-thought, let it be a conversation between two people who will die. Treating it as symptomatic ends the adolescent's reality-talk. (b) Depressive: the anhedonic register ("I'd rather not"), the loss of appetite for life rather than a question about it. Obligation: assess for depression — sleep, appetite, concentration, hopelessness, duration, function — and treat or refer accordingly; the existential conversation returns when the depression lifts, and can continue alongside it. (c) At risk: "won't be a problem for much longer" is a statement about the future absence of the speaker. Obligation: immediate risk assessment per procedure — intent, plan, means, previous attempts, current safety, who knows — and safety planning and escalation as CAC. 709 and CAC. 703 require. The three sentences differ by a clause; the obligations differ by a world. Practitioners who cannot hear the clause are dangerous.

> **SAFEGUARDING AND DUTY TO REPORT**
> Any statement that implies the speaker's future absence is a risk-assessment trigger, whatever its philosophical packaging. Assess directly, in plain words, the same session: are you thinking of killing yourself? Direct questioning does not implant the idea and does not increase risk; evasion does both. Follow your jurisdiction's and placement's procedure for recording, informing and escalating, and remember that confidentiality with a minor has stated limits which the young person should already know.

Lecture II — Meaning in adversity: the three routes

Frankl's account was written from the camps and must be used without romanticising them; what survives scrutiny is the structure. Where circumstances cannot be changed, meaning remains accessible through three routes, and each translates into conversational moves with a young person.

Creation — what the child makes and does. Work, art, care of another, a team, a garden, a channel, a protest. The question is not "what do you enjoy?" but "what is yours to do, that would not be done if you did not do it?" For a young person in a care home or a hospital ward, the answer is smaller and more real than any ambition: the Tuesday group that only happens because they turn up.

Experience — what the child receives and loves. Beauty, humour, friendship, a piece of music, being known. The route most attacked by depression and most defended by relationship: the counsellor's job is often to protect one small experience a week from the timetable that would cancel it.

Attitude — the stance toward what cannot be changed. The diagnosis that will not reverse, the parent who will not return, the sentence already served. Here the work is neither acceptance-as-resignation nor fighting, but the discovery that a position remains available: how this is borne, whom it is borne for, what it is made to testify to. This route is where existential counselling meets the third wave's acceptance (week 7) and the two lineages can be taught together without confusion: ACT supplies processes, existentialism supplies the seriousness.

> **PRACTICE NOTE**
> **Clinical implication.** Never use the attitude route to flatter a child into bearing what an adult should change. Where adversity is removable — poverty, bullying, an unsafe placement — the meaning conversation is a consolation and the removal is the treatment. Know which you are offering.

Worked example 9.2 — A meaning conversation that is not reassurance

Task. Zainab (16) has sickle cell disease with a third admission this term; she says, "My body is a prison and everyone's just waiting for me to be grateful it isn't worse." Compose the shape of a response using the three routes, and name what you must not say.

Solution. Acknowledge first: the prison is real, the gratitude demand is real, and her anger at it is accurate. Creation: what is hers inside the ward and after — the physics she tutors younger patients on WhatsApp, which stops when she stops. Experience: what survives admission — the playlist, the cousin who visits, the night nurse who doesn't do the gratitude speech. Attitude: not "be brave" but her own position — she has already chosen one, refusing to be inspirational, and that refusal is a stance with dignity in it that can be named and owned: "you've decided what this will not be; what have you decided it will be?" Forbidden: "at least it isn't worse", "everything happens for a reason", "you're so strong", and any version of the gratitude demand she has just told you about. The test of the response is whether Zainab's reality is still in the room at the end of it.

Lecture III — What the lineage refuses, and what it cannot do

Existential counselling refuses three things that the rest of the course supplies, and the refusal is its contribution. It refuses **technique-as-answer**, insisting that some material is met rather than treated. It refuses **normalisation-by-category**, insisting that this adolescent's death-anxiety is not an instance of a disorder but a person meeting a given. It refuses **the tyranny of adjustment**, insisting that a young person at odds with an unjust situation may be accurately at odds, and that the counsellor's loyalty is to the person rather than to the situation's smooth running — a position with obvious kinship to week 12′s critique.

What it cannot do is equally clear. It is not a treatment for disorder: depression, psychosis, trauma reactions and anxiety disorders require the methods of weeks 5 to 7 and 15 to 21 of the programme, and existential stance is the ground on which those methods are delivered humanely, not a substitute for them. It is developmentally limited: below early adolescence the givens are not yet available as objects of reflection, and work with children proceeds through play, relationship and changed circumstances. And it is culturally situated: the givens as formulated are a particular (largely European) philosophical inheritance, and other traditions frame death, self and meaning differently — which is precisely why week 12 follows this one.

PRACTICE NOTE

Clinical implication. Keep the lineage in its place: existential counselling is the room's floor, not its furniture. When a technique is needed, use it; when a given is spoken, put the technique down.

SKILLS LABORATORY

Practical 9 — the meaning interview. In pairs, twenty minutes. One plays an adolescent who has lost something real and unchangeable (cards supplied: a diagnosis, a death, a migration, a conviction in the family). The other conducts a meaning conversation: acknowledge the given without reassurance; explore one creation route, one experience route; invite an attitude position in the young person's own words; close by asking what they want the adult world to stop saying. Observer codes: any reassurance? any interpretation? any hurry? whose words ended the session? Swap and debrief.

ASSESSMENT

Assessed task. A transcript analysis and composed response of 1,000 words: classify three supplied utterances (philosophical, depressive, at-risk) with the linguistic and affective evidence for each, state the obligation each creates, and write the opening of your response to the at-risk utterance including the direct question you would ask.

Criterion	Weight
Classifications justified from the wording and affect	30%
Obligations stated correctly, including procedure for risk	30%
Composed response acknowledges before it does anything else	20%
Direct risk question present and plain	10%
Prose quality	10%

Formative. Bring one question of this kind that a young person has asked you, and what you actually said.

UNIT SYNTHESIS

Synthesis. The existential lineage teaches that some material is met rather than treated, that the adolescent's big questions are usually developmental rather than diagnostic, and that the practitioner's first competence with them is the capacity to hear exactly which of the three cases — philosophical, depressive, at-risk — is in the room.

READING (APA 7TH)

- Frankl, V. E. (2006). *Man's search for meaning*. Beacon Press. (Original work published 1946)
- Schneider, K. J., & May, R. (1995). *The psychology of existence: An integrative, clinical perspective*. McGraw-Hill.
- van Deurzen, E. (2010). *Everyday mysteries: A handbook of existential counselling* (2nd ed.). Routledge. — **core text**; the method chapters.
- Wong, P. T. P. (2010). Meaning therapy: An integrative and positive existential psychotherapy. *Journal of Contemporary Psychotherapy, 40*(2), 85–99.
- Yalom, I. D. (1980). *Existential psychotherapy*. Basic Books.
- van Deurzen, E., & Arnold-Baker, C. (Eds.). (2005). *Existential perspectives on human issues: A companion for therapeutic practice*. Palgrave Macmillan.

Seminar questions

1. Why is reassurance a refusal of company? Give the mechanism and one alternative first sentence.
2. A school refers a sixteen-year-old for "morbid preoccupation with death" on the evidence of poetry. What is your position, and what would change it?
3. Where is the line between tragic optimism and collusion with an unchangeable-but-removable adversity? Give a case.
4. The givens are a European inheritance. What is lost and what is gained by teaching them in a multicultural programme?
5. What did you do with the young person's question in your formative task, and what would you do now?

Next week

Week 10 changes level of analysis from the person to the system: Bateson, circular causality, and Minuchin's structural family therapy. Read the description of boundaries, subsystems and enactment before the session and bring one family in which the identified child was not the problem.

10 LECTURE UNIT 10 Teaching week 10 4 notional hours

The Systemic Lineage I: From Mind to System — Bateson, Circularity, and Structural Family Therapy

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Explain** the shift from linear to circular causality and reformulate one linear case statement as a loop.
2. **Draw a structural family map** showing subsystems, boundary quality (clear, enmeshed, disengaged) and any cross-generational coalition.
3. **Describe** joining and enactment as Minuchin's two principal techniques, and demonstrate an enactment in role-play.
4. **State** the ethical limits of systemic thinking with children: what it must never be used to say about abuse, and who is responsible for what.
5. **Plan** systemic-informed work when the family will not attend, using whoever is in the room.

Where this week sits

Every lineage so far has located the problem inside the child — in drives, contingencies, cognitions, conditions of worth, or the meeting of givens. This week moves it into the space between people, and the move is not an addition to the previous weeks but a change of level: the same child can be formulated intrapsychically, behaviourally and systemically at once, and a practitioner who cannot move between levels will keep treating the wrong unit. Daniel returns this week for the third time, and his family will finally be in the room.

Key concepts

CIRCULAR CAUSALITY

The claim that behaviour in a family is organised in loops rather than chains: each person's move is a response to the previous move and a stimulus for the next, so "who started it" is usually unanswerable and always unhelpful. In the room: the question "what happens next, and who notices first?" replaces "why did you do that?"

BOUNDARIES

The rules about who is in on what: between parents and children, between siblings, between family and outside. Clear boundaries permit both belonging and separateness; enmeshed ones permit no private position; disengaged ones permit no recourse. In the room: the family map's principal diagnostic, and the target of structural work.

ENACTMENT
Minuchin's technique of asking the family to do in the session what they do at home — "tell your mother now what you told me" — so that the structure is observable and available for change rather than reported. In the room: the difference between a family described and a family present.

IDENTIFIED PATIENT
The member who carries the symptom and, often, a function: the child whose refusal keeps two parents in contact, whose wetting keeps a grandmother in the house, whose rage keeps everyone from the grief. In the room: the reason treating the child alone so often fails, and the reason systemic language must be used with care — a function is not a blame.

Lecture I — From the mind to the loop

Proposition 1. The cybernetic turn made the relationship the unit. Bateson and colleagues' work on communication — including the double bind hypothesis, whose specific schizophrenia claims did not survive testing but whose insight about contradictory messages in trapped relationships endured — relocated pathology from the individual to the pattern of messages between individuals. What survived is methodological: observe the sequence, not the person.

Proposition 2. Linear causality produces blame; circular causality produces a map. "The mother's anxiety makes the child refuse school" is linear and indicting. The loop is different and larger: the child refuses, the mother's anxiety rises and she presses and stays close, the father withdraws from the conflict, the child's refusal deepens to hold the mother's attention and to spare the parents their quarrel, the school exempts, the loop tightens. Nobody is the author; everybody is a participant; the loop is the target. Figure 10.2 draws the generic form.

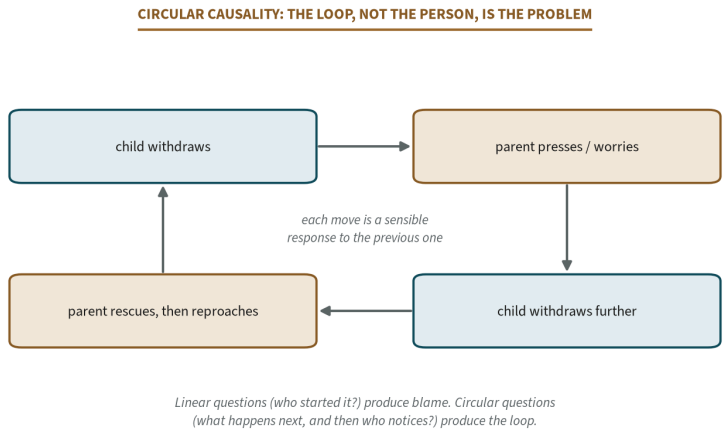


Figure 10.1 Circular causality as a four-node loop: the child withdraws, the parent presses and worries, the child withdraws further, the parent rescues and then reproaches, and the cycle repeats. Each move is a sensible response to the previous one; the loop, not the person, is the problem. *Source:* prepared for this edition.

Proposition 3. Symptom and system can stand in a stabilising relationship. Homeostasis is the observation that families resist change, including beneficial change, because the current arrangement solves something: the child's symptom may be regulating distance, detouring conflict, or maintaining a coalition. This is the most dangerous idea in the week, because misused it becomes "the child is doing it on purpose to hold the family together" — which is false, unjust and harmful. The accurate form is: the symptom has been recruited by the organisation, without anyone's intention, and the organisation will push back when the symptom is removed. Prepare for the pushback; do not accuse the child of it.

Proposition 4. Circular questioning is the assessment instrument. Asking each member what they observe between two others ("when your parents argue, what does your brother do?") generates the map faster and more safely than asking anyone about themselves, and it begins the intervention, because hearing the loop described is often the first time the family has seen it.

> **PRACTICE NOTE**
> **Clinical implication.** Before any family session, write the loop you hypothesise in four or five arrows. If you cannot, you will collect stories in the room rather than observe a system, and the loudest narrator will set your formulation.

Worked example 10.1 — Reformulating the linear statement

Task. A referral states: "Chidera (12) is manipulative and divides her parents; the father is weak and the mother is overbearing." Reformulate as a loop and state what the reformulation changes about the first session.

Solution. Loop: the parents disagree about discipline and do not resolve it (their conflict is undiscussable); Chidera's requests and complaints travel between them, and each parent's separate deal with her is a move in their marital conflict (coalition across generations); the father's withdrawal reads to the mother as abandonment, so she intensifies her involvement with Chidera (enmeshment); Chidera's escalating demands are the only channel through which the parents interact (symptom function); each escalation confirms the parents' view of each other (weak/overbearing), tightening the loop. "Manipulative" disappears: she is the conduit, not the author. First session changes: invite all three; do not begin with Chidera's behaviour; ask each what happens between the other two when a rule is disputed; draw the map live on paper where the family can correct it; and protect Chidera from the position of being the family's explanation — which is also the position that keeps her symptomatic.

Lecture II — Structural family therapy: subsystems, boundaries, enactment

Minuchin's contribution was to make the system treatable in the room rather than merely describable. Three elements carry the method.

The structural map. Subsystems (parental, sibling, spousal), boundaries and their quality, and coalitions — especially the cross-generational coalition in which a parent and a child are aligned against or apart from the other parent. Figure 10.1 draws Daniel's family in these terms, and the drawing is the diagnosis: an enmeshed mother–child dyad, a disengaged father, and a school in coalition with the dyad across the generation of professionals.

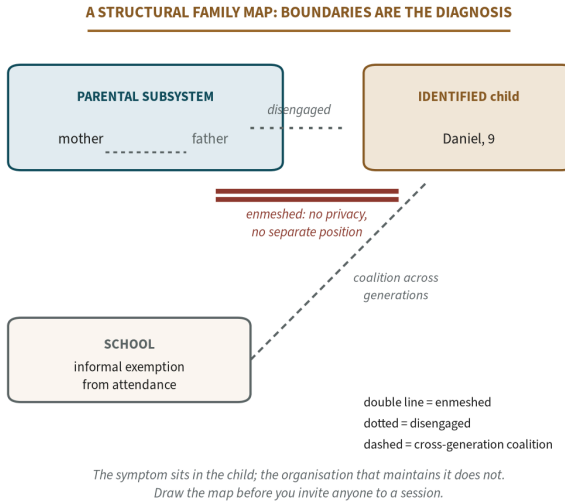


Figure 10.2 A structural family map of Daniel's family: parental subsystem with a disengaged father, an enmeshed mother-child dyad drawn with a double line, and a dashed cross-generational coalition with the school's informal exemption from attendance. Boundaries are the diagnosis; the symptom sits in the child but the organisation that maintains it does not. *Source:* prepared for this edition.

Joining. The therapist enters the family's style before challenging it: accommodates to their idiom, affirms each member's position, tracks their language. Joining is not warmth as manner; it is the purchase that makes the next move survivable. Without it, interventions are experienced as invasion and the family closes or leaves.

Enactment and boundary making. The therapist asks the family to perform the problematic sequence in the session and then alters it live: "tell your father now what you just told me"; "mother, let them finish without helping"; "father, what would you say to your son about this, if it were only the two of you?" The mechanism is experience: a new interaction, witnessed and repeated, changes the structure more durably than an explanation of it. Unbalancing — siding deliberately with the peripheral member to shift the hierarchy — is the advanced form and requires joined-ness first.

SAFEGUARDING AND DUTY TO REPORT

Systemic formulations must never be used to distribute responsibility for abuse. Where a child is harmed by an adult, the perpetrator is responsible, full stop; circular language applied to violence or sexual abuse ("the family co-constructed the context") is a professional failure with real victims. Systemic thinking explains organ-

isation, maintenance and recovery; it does not explain away harm, and safeguarding duties run to the child individually regardless of any family formulation.

Lecture III — When the family will not come

The pure model assumes the family in the room. Reality, especially in online and school-based practice, is frequently the child alone, or one parent. Four adaptations preserve the lineage without the full cast.

Work the loop with whoever attends. The child can report sequences; the loop can be drawn with the child and corrected; the child's own moves in the loop can change, and a change in one position alters the whole pattern. This is not second-best; it is systemic work at one node.

Use the telephone and the letter. A ten-minute call to a non-attending parent to ask a circular question ("what does your wife do when Daniel cries in the morning?") both gathers data and perturbs the system: the question travels home.

Brief the professionals in the loop. Teachers and carers are participants; changing the school's response to refusal (week 5) is systemic intervention by other means.

Hold the absent in the room. An empty chair, a genogram, a letter written and not sent: the absent member's position is represented so that the child is not the sole carrier of the family's story.

PRACTICE NOTE

Clinical implication. A child alone in a room is still a member of a system, and your questions can be systemic even when your seating is not. "What happens at home just before it gets worse?" is a systemic question asked of one person.

Worked example 10.2 — An enactment, scripted

Task. In session with Daniel and both parents (father attending for the first time), you hypothesise that the mother answers for Daniel and the father has no position. Write the enactment you would set, the sentence you would use to initiate it, and two things you would watch for.

Solution. Initiation, after joining with each: "Daniel, you told me last week what the morning feels like. I'd like you to tell your dad now, here, while your mum and I listen. Dad, your job for the next five minutes is only to listen and then say back what you heard — not to fix it." Watch for: (1) the mother's first intervention — its timing and form are the enmeshment datum, and when it comes you name it gently and return the floor to Daniel ("mum, I know the answer; let's hear his"); (2) the father's capacity to reflect rather than fix or dismiss — his first accurate reflection is the session's change event, and is marked and repeated ("say that again to him; that's the sentence he hasn't heard"). The enactment converts

the map into an experience, and the experience — not the map — is what the family takes home.

SKILLS LABORATORY

Practical 10 — map and enactment. In fours. One supplies a family (card or anonymised experience); the group draws the structural map in ten minutes (subsystems, boundary qualities, coalition) and agrees one hypothesis. Then a fifteen-minute role-play: two as parents, one as child, one as therapist who must join, then set one enactment and intervene once in it. Debrief: was the joining real or performed? did the enactment target the hypothesis? who in the family was protected by the therapist's move?

ASSESSMENT

Assessed task. A structural formulation of 1,200 words on a supplied family vignette: the map drawn and justified from the text, the loop written in arrows, one enactment specified with its initiating sentence and its target, and a paragraph on what the formulation must not be used to say.

Criterion	Weight
Map justified from the material; boundary qualities named	30%
Loop written circularly; no linear blame	25%
Enactment targets the hypothesis and is survivable	25%
Ethical limits paragraph accurate	10%
APA 7th and prose quality	10%

Formative. Bring one family in which the identified child was not the problem, with the observation that showed it.

UNIT SYNTHESIS

Synthesis. The systemic lineage changes the unit of treatment from the person to the pattern: circular causality replaces blame, boundaries replace traits, and enactment turns the family's structure into an experience that can be altered in the room — with the absolute limit that no formulation distributes responsibility for harm.

READING (APA 7TH)

- Bateson, G., Jackson, D. D., Haley, J., & Weakland, J. (1956). Toward a theory of schizophrenia. *Behavioral Science, 1*(4), 251–264.
- Carr, A. (2019). *Family therapy: Concepts, research and practice* (3rd ed.). Wiley.
- Haley, J. (1976). *Problem-solving therapy: New strategies for effective family therapy*. Jossey-Bass.

- Minuchin, S. (1974). *Families and family therapy*. Harvard University Press. — **core text**; structure, boundaries and the technique chapters.
- Nichols, M. P., & Davis, S. D. (2020). *Family therapy: Concepts and methods* (12th ed.). Pearson.
- Vetere, A. (2001). Structural family therapy. *Child Psychology and Psychiatry Review, 6*(3), 133–139.

Seminar questions

1. "The symptom stabilises the system" — state the claim precisely, then state the three ways it is misused in practice.
2. Why must joining precede enactment? Describe a session in which the order was reversed.
3. A parent says systemic therapy "blames the family for what the child does". Compose the correction.
4. What is lost when systemic work is done with the child alone, and what is unexpectedly gained?
5. Where does the systemic level conflict with the safeguarding level, and which governs? Give a case.

Next week

Week 11 completes the systemic lineage with its two post-modern children: solution-focused brief therapy and narrative therapy. Read the scaling question and the externalising conversation before the session and bring one example of a child's own name for their problem.

11 LECTURE UNIT 11 Teaching week 11 4 notional hours

The Systemic Lineage II: Solution-Focused and Narrative Practice with Children

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **Conduct** a scaling conversation, including the coping question and the "what told you it was a 4 and not a 3?" question, and explain why the latter is the intervention.
2. **Externalise** a problem with a child in the child's own naming, and map the problem's tricks and the child's counters.
3. **Identify** unique outcomes and use one to begin a re-authoring conversation with an audience.

4. **State** the evidence position for SFBT and narrative therapy with young people, including where claims outrun data.
5. **Explain** what these approaches share with, and refuse from, the problem-centred lineages of the course.

Where this week sits

Week 10 treated the family as the unit and the problem as real. This week's two approaches make a different wager: that detailed analysis of the problem is not necessary for change, and that the story a child tells about themselves is not a report but a construction that can be re-authored. Both grew from the systemic lineage's discovery that language and interaction constitute the problem; both then went further than Minuchin would follow. Used well they are among the most respectful methods available for work with children; used lazily they are a way of not listening.

Key concepts

THE SCALING QUESTION
A 0–10 scale in the child's own metric, used not to measure but to discover: what is already working, what the next small step is, and what evidence the child already has of movement. In the room: "what told you it was a 4 and not a 3?" — because the answer lists the child's own competencies, in the child's words, and lists them as facts rather than compliments.

EXTERNALISATION
The narrative move of separating the child from the problem: the problem becomes an entity with habits, tricks and intentions that can be described, resisted and outwitted. In the room: "the Worry" rather than "your anxiety" — a grammatical change with clinical consequences, because a child can fight a character in a way they cannot fight a trait.

UNIQUE OUTCOME
An event that the dominant problem-saturated story cannot account for: the night the Worry came and was not obeyed. In the room: the raw material of the alternative story, and easily missed because it contradicts the narrative everyone has come to believe.

RE-AUTHORING WITH AN AUDIENCE
The deliberate recruitment of witnesses to the alternative story, because a story held by one person is a rumour and a story held by several is an identity. In the room: who needs to be told that she outwitted the Worry on Tuesday, and what will they say?

Lecture I — Solution-focused practice: the problem is not the necessary route

Proposition 1. Solutions do not require analysis of the problem. de Shazer and colleagues' radical claim is that knowing the aetiology is not necessary for constructing a solution; what is necessary is amplifying what already works. The corollary methods: the miracle question (what would be the first small sign, tomorrow, that the problem had gone?), exception questions (when was it slightly less bad, and what was different?), scaling, and coping questions (given how hard this is, how have you managed to keep going at all?).

Proposition 2. The child is the expert on their own scale. Scaling appears to be measurement and is actually respect: the metric, the anchors and the movement are the child's. Figure 11.1 draws the conversation; the therapeutic action sits in the gap between 4 and 3, which belongs to the child.

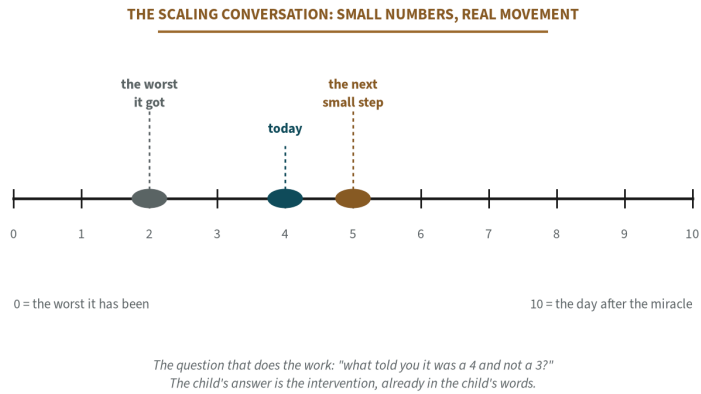


Figure 11.1 The scaling conversation on a zero-to-ten line: the worst it got, today, and the next small step, with the question 'what told you it was a four and not a three?' whose answer is the intervention already in the child's words. *Source:* prepared for this edition.

Proposition 3. Small is the unit. SFBT works in increments the child can actually take this week, and treats the increment as the whole achievement rather than a step toward a professional goal. With children this is not modesty but accuracy: change in a child's life is made of small things repeated in small rooms.

Proposition 4. The evidence is real and bounded. Reviews of SFBT outcome research — Gingerich and Eisengart's frequently cited review among them — find a positive signal across studies of variable quality, with stronger effects in some child and school settings; the method is brief, acceptable and cheap. What the evidence does not support is superiority over active alternatives, nor its use as a substitute for assessment where risk or disorder is present. SFBT is an excellent first conversation and an excellent whole therapy for some presentations; it is not a risk assessment and must not be used as a way of avoiding one.

> **PRACTICE NOTE**
> **Clinical implication.** Resist the professional hunger for the story behind. In a solution-focused frame, asking "why" is not depth; it is a change of method. If you need the history this week, say so and take it elsewhere in the session — do not smuggle it into the scaling conversation.

Worked example 11.1 — A scaling conversation, transcribed in shape

Task. Emeka (10), referred after his parents' separation, rates "the shouting inside" at 3 today; two months ago it was 8. Write the four questions you would ask, in order, and what each is for.

Solution. (1) "What told you it's a 3 today and not a 2?" — discovers what is working: he names that he sleeps at his cousin's on Wednesdays and that he has stopped waiting by the phone. These are his competencies and his exceptions, recorded as facts. (2) "What would be the first tiny sign, a week from now, that it had moved to a 4?" — builds the next small step in observable terms: "I'd eat breakfast without my aunt asking twice." (3) "Back when it was an 8 and everything was worse — how did you manage to keep going at all?" — the coping question, which surfaces strength under the worst conditions and dignifies survival: "I drew. And I made sure my little brother wasn't in the room." (4) "Who has noticed that it's a 3? What did they say?" — recruits the audience and tests whether the change is socially real. Note the order: competence, next step, survival strength, witness. Note also what was never asked: why the parents separated, and how he felt about it. That history may matter later; it is not this method's door.

Lecture II — Narrative practice: the story is the treatment

Proposition 1. The dominant story organises experience. A child described and describing themselves as "the bad one" will notice, remember and expect evidence for the story, and the story will recruit adults as its evidence-gatherers. The problem-saturated story is not a distortion to be disputed (week 6) but a narrative to be rivalled.

Proposition 2. Externalisation creates the fighting position. "The person is not the problem; the problem is the problem." Once the Worry, the Rage or

the Black Dog has habits and intentions, the child can report on it, resist it, and — decisively — be morally repositioned: failures become the problem's victories rather than the child's defects. Figure 11.2 draws the separation and the audience that makes it stick.

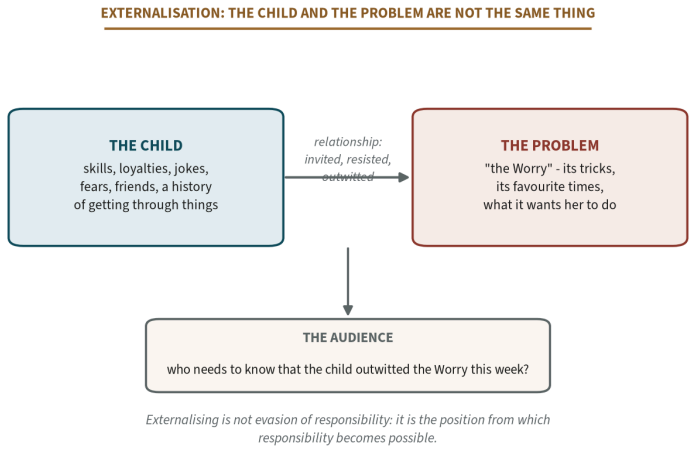


Figure 11.2 Externalisation as the separation of the child and the problem into two boxes with a describable relationship between them — the problem's tricks, the child's counters — and an audience of witnesses below, because a story held by several people is an identity rather than a rumour. *Source:* prepared for this edition.

Proposition 3. Unique outcomes are the seed of the alternative story. The search is deliberate and forensic: across the week, when did the Rage come and lose? Each exception is then thickened — who was there, what did it say about you, what would your grandmother call that? — until the alternative story has events, witnesses and a trajectory.

Proposition 4. With children, narrative method becomes play and document. The problem is drawn, given a size and a silly voice, interviewed ("Worry, what do you want for Chidi?"), and outwitted in planned operations. Letters and certificates from the counsellor, and definitional ceremonies in which chosen witnesses declare what they saw, do with paper and ritual what adult narrative therapy does with conversation. The documents are not decoration: for a child, an identity held in a letter in a school bag is durable.

> **SAFEGUARDING AND DUTY TO REPORT**
> Externalisation has one strict limit: it must never be applied to responsibility for harm done to others. "The Rage made me hit him" is not an externalisation; it is an evasion wearing the method's clothes. Where a child has harmed, the work separates

the child's identity from the problem while holding the child's responsibility for the act plainly — both at once, in the same sentence, without contradiction.

Lecture III — What the two share, what they refuse, and what the evidence allows

SFBT and narrative practice share a constructionist premise — problems are constituted in language and interaction — and a posture of not-knowing: the child is the expert, the practitioner is a curious editor rather than an interpreter. They refuse diagnosis, developmental tables and the analytic authority of the previous weeks; and they refuse, deliberately, the idea that the practitioner knows the goal better than the child.

The refusals carry costs that this course names plainly. A not-knowing stance in the face of a depressed fourteen-year-old who cannot generate exceptions is not respect but abandonment of clinical responsibility; where disorder, risk or safeguarding is present, the assessment obligations of weeks 3, 4, 9 and 12 govern and the methods of this week resume afterwards. The evidence, too, is asymmetric: SFBT has an outcome literature of mixed but genuine quality; narrative therapy's is thin, consisting largely of single cases and practitioner reports, so its claims should be held as a well-developed practice tradition rather than as an evidence-based treatment in the week 14 sense. Neither deficiency makes the methods worthless; both make honesty about them obligatory.

> **PRACTICE NOTE**
> **Clinical implication.** Borrow freely across lineages but declare the borrowings. A scaling question inside a CBT session is fine; calling the whole session solution-focused when you also ran a risk assessment and a cognitive experiment is not accuracy, it is costume.

Worked example 11.2 — The child's own name for the problem

Task. Funmi (8) calls her nightly terror "the Night Man". Using her naming, sketch the externalising conversation: three questions about the problem's habits, two about her counters, and one audience question. Then state what you would do with the parents.

Solution. Habits: "When does the Night Man come — what time, and what does he do first?" "What does he tell you about your mum and dad?" "What does he want you to do, and what happens if you do it?" Counters: "Think of a night he came and you didn't do what he wanted. What did you do instead?" "Who helped you that night, even a little?" Audience: "If the Night Man lost three nights in a row, who would be the first person who'd notice, and what would they say?" With the parents: teach the grammar — the Night Man is the adversary, Funmi

is not; their job is to be on her side of the fight and to notice and report victories aloud, because a counter that nobody witnesses does not become identity. The naming is hers and must not be improved; "night terror" is your word and stays in your notes.

SKILLS LABORATORY

Practical 11 — scaling and externalising laboratory. In triads, rotating. Round A (eight minutes): a full scaling conversation on a supplied card, including coping question and audience question; observer notes whether any "why" slipped in. Round B (eight minutes): externalise the card's problem using the "client's" own naming, map two tricks and two counters, and recruit one audience member. Debrief: which questions produced new information, and which produced politeness?

ASSESSMENT

Assessed task. A recorded ten-minute solution-focused or narrative segment with a peer (your choice of frame), submitted with an 800-word commentary: the frame declared, the questions listed by type, one moment where the problem-saturated story reasserted itself and how you handled it, and one sentence on what the frame cannot see in this case.

Criterion	Weight
Frame declared and held; question types identifiable	30%
Child's (peer's) language and metric used, not the practitioner's	25%
Handling of the problem-saturated re-assertion	20%
Honest statement of the frame's blind spot	15%
Prose quality	10%

Formative. Bring one example of a child's own name for their problem, and what the naming made possible.

UNIT SYNTHESIS

Synthesis. Solution-focused and narrative practices wager that change does not require the problem's full analysis — that competencies can be amplified and stories re-authored in the child's own language — and their discipline is knowing when the wager must yield to assessment, risk work and the other lineages.

READING (APA 7TH)

- de Shazer, S. (1985). *Keys to solution in brief therapy*. W. W. Norton.
- Gingerich, W. J., & Eisengart, S. (2000). Solution-focused brief therapy: A review of the outcome research. *Family Process, 39*(4), 477–496.
- Morgan, A. (2000). *What is narrative therapy? An easy-to-read introduction*. Dulwich Centre Publications.
- White, M., & Epston, D. (1990). *Narrative means to therapeutic ends*. W. W. Norton. — **core text**; externalisation and re-authoring.
- Berg, I. K. (1994). *Family-based services: A solution-focused approach*. W. W. Norton.
- Winslade, J., & Monk, G. (2000). *Narrative counseling in schools: Powerful and brief*. Corwin Press.

Seminar questions

1. Is "what told you it was a 4 and not a 3?" a cognitive technique? Defend your answer against week 6.
2. A child externalises so thoroughly that responsibility for harm evaporates. Reconstruct the sentence that holds both.
3. What does the not-knowing stance protect the child from, and what does it protect the practitioner from?
4. Narrative therapy's evidence base is thin. Should it be taught and used? On what terms?
5. Where in your own practice does the hunger for the story behind interfere with the conversation in front of you?

Next week

Week 12 turns the course's attention onto the course: multicultural, feminist and decolonial critiques of every lineage studied so far. Read the multicultural guidelines and bring one moment in which your own position in the room was the first thing the child experienced.

12 LECTURE UNIT 12 Teaching week 12 4 notional hours

Critique: Multicultural, Feminist and Decolonial Readings of Counselling Theory

LEARNING OUTCOMES

By the end of this week you will be able to:

1. **State** for each lineage studied so far one way in which its construction reflects the population on whom it was built, and one consequence for a child outside that population.
2. **Complete a cultural formulation** of a case across the four quadrants, including the quadrant about the practitioner.
3. **Identify** two documented biases in assessment and diagnosis of Black and minority children, and state what a practitioner can and cannot do about them.
4. **Write a positionality statement** and use it to analyse one of your own recorded or recalled sessions.
5. **Evaluate** the claim that evidence-based practice is a form of governance, stating what survives the critique.

Where this week sits

Eleven weeks of theory, and not once have we asked who the theory was built on, by whom, and at whose expense. That question is not an add-on to the course; it is a condition of using the course honestly. The lineages you have learned were constructed overwhelmingly on, and by, people of European descent, in high-income countries, in clinical and university settings, with assumptions about family, self, emotion and authority that are local rather than universal. This week does not discard them. It teaches you to use them with the knowledge of their provenance, which is the difference between a tool and a prejudice.

Key concepts

CULTURAL HUMILITY
The stance of ongoing self-evaluation and curiosity about difference, as distinct from cultural competence conceived as a possessed inventory of facts about groups. In the room: the difference between "I know about Yoruba families" and "I do not know what being Yoruba, Christian, and twelve in this city means to you — teach me the parts that matter here."

POSITIONALITY
The practitioner's own social location — race, ethnicity, gender, class, faith, language, age, disability, sexuality, migration status — and the power it carries into the room before a word is spoken. In the room: the first thing the child experiences, and the thing least examined by the practitioner.

INTERSECTIONALITY
Crenshaw's account that systems of disadvantage do not add but intersect: a poor Black girl in state care is not girl plus Black plus poor but a distinct position produced by their interaction, which single-axis analysis and single-axis services cannot see. In

the room: the reason "the family" you keep failing to engage is not one family-shaped problem.

IDIOM OF DISTRESS
The culturally shaped form in which suffering is expressed and recognised: somatic complaint, spiritual experience, behavioural collapse, silence, possession idioms, anger. In the room: the reason a diagnosis can be both accurate and wrong — accurate to the manual, wrong to the person — and the reason referral thresholds misfire across cultures.

Lecture I — The theories were built on somebody

Proposition 1. The samples are local. Attachment research's foundational cohorts were American and Ugandan infants in particular conditions; CBT's trials over-represent white, middle-class, insured or university-adjacent families; the person-centred conditions were formulated in a mid-century American idiom of selfhood; systemic theory generalised from European and North American family forms in which the nuclear household and the speaking-of-feelings are norms. Each claim travels better than its critics allow and worse than its authors assumed.

Proposition 2. The self the theories address is a particular self. The autonomous, verbally reflective, individually bounded self who "has" feelings and "holds" beliefs is not the universal human but a cultural product. In many of the worlds your clients inhabit, the self is relational before it is individual: distress is a family event, healing is a communal process, and an intervention that isolates the child for individual insight can be experienced as an amputation. Figure 12.1 sets out the four questions that keep this in view.

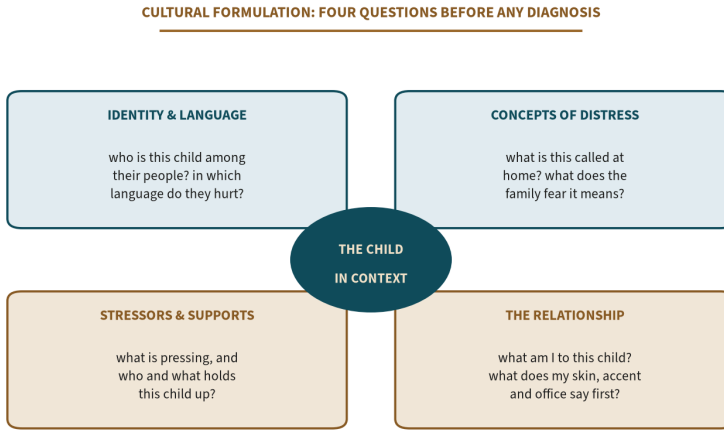


Figure 12.1 Cultural formulation as four quadrants around the child in context: identity and language, cultural conceptualisations of distress, psychosocial stressors and supports, and the relationship — what the practitioner's own skin, accent and office say first. After the cultural formulation interview structure; the fourth quadrant is the one practitioners skip and the one the child experiences first. *Source:* prepared for this edition.

Proposition 3. The developmental tables are cultural artefacts too. Milestones, "age-appropriate" behaviour, attachment categories and even the concept of adolescence are calibrated on particular populations. A nine-year-old who cooks for siblings, translates for parents, or cares for a grandmother is not parentified by every valid reading; some of those readings are class and cultural prejudice wearing a diagnostic coat.

Proposition 4. None of this makes the theories useless. It makes them hypotheses with a provenance. The mechanisms they name — reinforcement, appraisal, regulation, relational expectancy, narrative — are observed across cultures; the forms they take and the meanings they carry are not. The competent practitioner uses the mechanism and investigates the form.

> **PRACTICE NOTE**
> **Clinical implication.** Before applying any instrument, manual or milestone to a child outside the population it was built on, ask: on whom was this calibrated, and what would differ here? If you cannot answer, say so in the record rather than implying a universality the evidence does not hold.

Worked example 12.1 — The accurate-but-wrong diagnosis

Task. Mr Osei (13, Ghanaian-born, in the country four years) is referred for "conduct problems": defiance, leaving class, an incident of striking a peer. School

records note "no remorse". His mother describes headaches and a burning sensation in his legs, and says he "has been told" by an elder that the family's trouble follows him. Teacher and mother disagree completely about what is wrong. Formulate across the four quadrants.

Solution. Identity and language: his strongest emotional vocabulary may be in a home language in which no assessment has been conducted; "no remorse" may be a register of respect (not meeting an accuser's eyes, not speaking in self-defence) misread as callousness. Concepts of distress: somatic and spiritually framed idioms are the family's legitimate grammar of suffering; a formulation that reads them as superstition has already lost the case. Stressors and supports: migration, a father absent through the visa system, a mother working nights, an elder who is both support and authority; the school is a stressor, not a neutral observer. The relationship: a white assessor with a checklist is, to this family, an institution with a history; the mother's disagreement is data about that history, not resistance. The conduct formulation may still be partly right — contingencies maintain the leaving and the striking (week 5) — but it is a description of behaviour without its meaning, and acting on it alone would punish a child for an idiom. The plan: assessment in his language where possible; a cultural formulation interview with the mother; behavioural analysis of the classroom sequence; and a meeting in which the elder's account is heard as an account rather than corrected.

Lecture II — Bias in assessment, and what a practitioner can do about it

Two biases are documented well enough that ignorance of them is negligence.

Over-identification in special education and conduct diagnosis. Black children — and in several systems, Black boys specifically — are over-represented in conduct disorder, emotional-and-behavioural-difficulty categories and exclusion statistics, and under-represented in autism and anxiety identification, at rates not explained by prevalence alone. The mechanism is not usually individual malice but the calibration of norms, the interpretation of affect and register, and the accumulation of small discretionary decisions. What you can do: use multiple informants and settings, use observation in more than one context, record the alternative explanations you considered, and audit your own referrals by category and ethnicity annually. What you cannot do: neutralise the system alone — which is a political and institutional task, and saying so is not an excuse.

Under-recognition of distress in minority and migrant children. Where idioms differ, internalising difficulty is missed until it presents as crisis or somatic emergency. What you can do: ask about somatic and spiritual experience directly and respectfully, use idioms-of-distress questioning in the cultural formulation,

and treat a parent's framing as a hypothesis to test rather than an obstacle to remove.

Feminist critique adds the gendered layer: the girl whose distress is "attention-seeking" or "dramatic", the boy whose depression is read as anger, the carer roles assigned to girls and then pathologised as parentification, and the historical use of diagnosis to discipline women and girls who refuse. Intersectionality is the instruction to hold these together rather than in separate columns.

> **SAFEGUARDING AND DUTY TO REPORT**
> Bias is a safeguarding issue: children are missed and children are mislabelled by the same mechanism, and both errors endure in records that follow them into adulthood. Where you inherit a file whose formulations carry unexamined cultural interpretation — "no remorse", "chaotic family", "poor engagement" — treat those phrases as hypotheses requiring re-evidence, not as findings.

Lecture III — Evidence-based practice as governance: the critique and what survives

The strongest version of the critique runs thus: what counts as "evidence" is decided by institutions with interests; the randomised trial privileges interventions that can be manualised and counted, which favours brief, individual, symptom-targeted treatments over relational, communal and structural work; the evidence base therefore shapes the services that get funded, the training that gets accredited, and ultimately the kind of suffering that gets recognised — a governance of distress in the language of science. Decolonial critique adds that the global export of these manuals reproduces a colonial pattern in which the North defines the South's minds, and local healing systems are positioned as folklore while imported protocols are positioned as medicine.

Three responses are available, and the honest position holds all three. First, the critique is substantially true about incentives and silences: the evidence base is not a mirror of what works but a map of what has been funded and measured. Second, the critique does not dissolve the obligation to prefer better-evidenced to worse-evidenced practice where a child is in front of you this afternoon; abandoning evidence because evidence is politicised abandons the child to fashion and to power exercised less visibly. Third, the resolution is practice-level and humble: use the evidence, name its provenance and its gaps in the record, measure your own outcomes with this child (week 14), and hold local knowledge as knowledge — which is what the cultural formulation is for.

PRACTICE NOTE

Clinical implication. Write one sentence in every formulation naming what you do not know about this child's world and how you intend to find out. The sentence is the audit trail of humility, and it is what a supervisor can actually check.

Worked example 12.2 — Positionality in one session

Task. Write a positionality statement of three sentences for yourself, then apply it: recall or role-play a first session with a child of a different gender, faith or class from you, and identify the first three moments your position was in the room.

Solution (model). Statement: "I am a [your descriptors] practitioner, trained in [lineages], working in [setting], and my accent, credentials and skin place me inside the institutions that have historically decided which children are disturbed. I have never been assessed by someone who could not pronounce my name, and I do not know what that does to a child's trust in the first ten minutes. My theoretical preferences reward verbal reflection, which is a cultural skill as well as a psychological one." Application: moment 1 — the child's greeting calibrated to an authority figure, not to me personally; I read it as guarded. Moment 2 — my opening question ("how does that make you feel?") assumed a feeling vocabulary and a legitimacy of feeling-talk that the child's household may not share; the silence that followed was my question's property, not the child's deficit. Moment 3 — my note afterwards said "poor eye contact", a norm of my culture written as an observation of his. Each moment is small; the file is made of them.

SKILLS LABORATORY

Practical 12 — the adaptation audit. In pairs, take one manualised protocol from weeks 5–7 (a graded exposure plan, a thought record, a scaling conversation). Task: identify three points at which the protocol assumes a cultural norm (language, family structure, autonomy of the child, concepts of emotion, time and punctuality, authority), and design one adaptation at each point that preserves the mechanism while changing the form. Criteria: the mechanism must survive; the adaptation must not become a different therapy; the child's community must be able to recognise themselves in it. Present one adaptation to the group.

ASSESSMENT

Assessed task. A critical review of 1,200 words: choose one lineage from weeks 2–11 and one population it was not built on. State what in the lineage travels, what does not, and what a practitioner must add; support with at least one empirical source on bias or adaptation and one source from the population's own scholarship where available.

Criterion	Weight
Provenance of the lineage stated accurately	25%
What travels and what does not, argued rather than asserted	30%
Practical additions specified at mechanism level	25%
Sources include scholarship from the population concerned	10%
Prose quality	10%

Formative. Submit your three-sentence positionality statement. It is not graded; it is used in supervision.

UNIT SYNTHESIS

Synthesis. The critiques do not retire the lineages; they date and place them. The practitioner's competence is to use each mechanism while investigating its form, to audit one's own position and one's own referrals, and to hold the evidence base as the best available map rather than the territory.

READING (APA 7TH)

- American Psychological Association. (2017). *Multicultural guidelines: An ecological approach to context, identity, and intersectionality*. APA.
- Crenshaw, K. (1989). Demarginalizing the intersection of race and sex: A Black feminist critique of antidiscrimination doctrine, feminist theory and antiracist politics. *University of Chicago Legal Forum, 1989*(1), 139–167.
- Fernando, S. (2014). *Mental health worldwide: Culture, globalization and development*. Palgrave Macmillan.
- Sue, D. W., & Sue, D. (2016). *Counseling the culturally diverse: Theory and practice* (7th ed.). John Wiley & Sons. — **core text**; the foundations and the skills chapters.
- Sue, D. W. (2001). Multidimensional facets of cultural competence. *The Counseling Psychologist, 29*(6), 790–821.
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Seminar questions

1. Is cultural humility a skill or a virtue? What would training each look like, and which can be examined?
2. A manual works in trials and fails in your clinic. List the five most likely reasons in order, and say how you would tell them apart.

3. Where does respectful use of a family's spiritual explanation end and collusive silence about harm begin? Give a case.
4. If the evidence base is a form of governance, what governs in its absence? Argue from a setting you know.
5. Which of your own positionality sentences was hardest to write, and what does its difficulty protect?

Next week

Week 13 begins integration: case formulation across competing accounts, and the discipline of holding four readings of one child at once. Re-read your notes on Daniel from weeks 1, 5 and 10 before the session; you will need all three.

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